IPMG

Deep-Research Study Briefs

— 2026 RECRUITMENT LIBRARY —

33 ACTIVE PROTOCOLS · 4 SITES

Chino (18) · Redlands (3) · Temecula (7) · San Juan Capistrano (5)

PREPARED FOR
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"Where Science Meets the Soul."

— Fady Boules, PMHNP-BC

IPMG RESEARCH · (909) 955-5865
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Executive Summary

2026 Recruitment Library · Corrected Edition

Thirty-three active protocols span four IPMG sites — Chino (18), Redlands (3), Temecula (7), and San Juan Capistrano (5) — forming the 2026 recruitment library.

Coverage includes Major Depressive Disorder (monotherapy, adjunct, TRD, and biomarker-matched), Bipolar I and II depression and mania, Schizophrenia (adult and adolescent), Alzheimer's psychosis and agitation, Post-Traumatic Stress Disorder, Generalized Anxiety Disorder, Binge Eating Disorder, Tardive Dyskinesia, pediatric bipolar, pediatric schizophrenia, autism irritability, and psychedelic / neuroplastogen programs (psilocybin, methylone).

Each study brief follows an 18-section format: Identification, Hook, Unmet Need, At-a-Glance, Why This Study Exists, Mechanism of Action, Study Design Deep Dive, Recruit If, Stop/Exclude, Ideal Patient Profile, eCW + ICD-10 search plan, Competitive Landscape, Risk/Benefit, Why This Matters for a Large Psychiatric Group, Provider Talk Track, 3-Step Recruit Move, Final CTA, and Sources.

Screening for every protocol routes through **IPMG Research at (909) 955-5865**.

This library is a working reference. Each brief ends with a specific comment code and a one-week screening ask. Save it, screen one patient per study per week, and the aggregate enrollment contribution across 33 protocols becomes category-defining for Southern California psychiatric research.

■ 2026 CORRECTION

v2.0 CORRECTIONS APPLIED: This edition contains eight factual corrections to study designs, eligibility criteria, target enrollment numbers, and sponsor attributions relative to v1.0. Corrected studies are flagged inline with an orange **■ 2026 CORRECTION** callout. Affected studies: 02 (NORLIGHT — TRD adjunct, not monotherapy), 03 (ALTOBIP — 6-week DB with 7-week OLE), 04 (CLARITY — randomized-withdrawal with OL lead-in), 08 (BALSAM — strictly adjunctive to Li/VPA/LTG), 14 (M4OLE — emraclidine is AbbVie), 24 (KORMDD — N~195, excludes ≥3 AD failures), 28 (DREEM3S — age 18–74), 30 (EMBRACE — N~330, 8/16 mg 3-arm).

— Fady Boules, PMHNP-BC · Where Science Meets the Soul.

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33	TSND-201 (Methylone) in PTSD — EMPOWER-1	**EMPOWER1**	NCT07456696

What Changed in v2.0

Eight factual corrections from v1.0 of the 2026 library

STUDY 02 · NORLIGHT (ACP-211 in MDD)

Corrected to **TRD adjunct design**. Patients must currently be on an approved antidepressant AND have documented history of ≥2 prior antidepressant failures in the current episode. AD-naive and washed-out patients are NOT eligible. Recruiting the right patients requires searching FOR refractory cases — not screening them out.

STUDY 03 · ALTOBIP (ALTO-100 in Bipolar Depression)

Corrected to **6-week double-blind period** (not 8 weeks) with primary MADRS endpoint at Week 6, followed by an **open-label period of up to 7 weeks**.

STUDY 04 · CLARITY (Solriamfetol MDD + EDS)

Corrected to **randomized-withdrawal design with open-label lead-in**. All eligible participants FIRST receive open-label solriamfetol. Only responders are then randomized 1:1 to continue drug or withdraw to placebo. Every patient is guaranteed active drug at the start — a material pitch point.

STUDY 08 · BALSAM (KarXT BALSAM-4 in Bipolar I Mania)

Corrected to strictly **adjunctive design**. Patients MUST currently be on — and remain on throughout the trial — **lithium, valproate, or lamotrigine**. KarXT or placebo is added on top. Do NOT screen patients looking to switch off their mood stabilizer.

STUDY 14 · M4OLE (NBI-1117568 Schizophrenia OLE)

Corrected sponsor attribution: **emraclidine is an AbbVie asset** (acquired via the $8.7B Cerevel deal), NOT a Bristol Myers Squibb asset. BMS owns the competitor, KarXT (Cobenfy). Emraclidine failed its Phase 2 schizophrenia trials in November 2024.

STUDY 24 · KORMDD (Icalcaprant MDD)

Corrected **target enrollment to ~195** (not ~500). Corrected exclusion criterion: patients are excluded only for ≥**3 prior antidepressant failures**. Patients with exactly 2 prior failures ARE eligible — the v1.0 brief would have screened them out incorrectly.

STUDY 28 · DREEM3S (Seltorexant Sleep EEG)

Corrected **age range to 18–74** (not 18–65). Older adults up to age 74 are eligible, substantially broadening the eligible pool.

STUDY 30 · EMBRACE (CYB003 Phase 3 MDD)

Corrected **target N to ~330** (not ~220) and **doses to 8 mg and 16 mg** (not 12/16 mg — the latter was APPROACH). EMBRACE is a 3-arm study (placebo vs 8 mg vs 16 mg) randomized 1:1:1, not a mirror of APPROACH.

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SITE

Chino

18 active protocols · 2026

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03	ALTO-100 in Bipolar Depression	ALTOBIP
04	Solriamfetol in MDD + Excessive Daytime Sleepiness (CLARITY)	CLARITY
05	BHV-7000 in MDD — Long-Term Safety	BHVLTS
06	BHV-7000 MDD Monotherapy	BHVMONO
07	KarXT in Adolescent Schizophrenia	COBENTEEN
08	KarXT BALSAM-4 in Bipolar I Mania	BALSAM
09	KarXT ADEPT-4 in Alzheimer's Psychosis	ADEPT4
10	KarXT + KarX-EC ADEPT-5 in Alzheimer's Psychosis	ADEPT5
11	BMS-986368 for Agitation in Alzheimer's	CALM45
12	NBI-1065845 (Osavampator) Adjunctive MDD	SAVITRI3
13	NBI-1065890 in Tardive Dyskinesia	NEXTVMAT
14	NBI-1117568 Schizophrenia Open-Label Extension	M4OLE

15	**Valbenazine TD4027 Residual TD**	**RESIDUALTD**
16	**SPT-300 (GlyphAllo) MDD with Anxious Distress — BUOY-1**	**BUOY1**
17	**SPT-300 MDD Open-Label Extension**	**BUOYOLE**
18	**RAP-219 in Bipolar I**	**RAPBP**

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CHINO · STUDY 01 · COMMENT CODE YOUTHLIFT

Cariprazine (Vraylar) in Pediatric Bipolar I Depression

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
3112-301-001	NCT04777357	AbbVie / Allergan	Chino	YOUTHLIFT

1 · THE HOOK

STILL watching a 13-year-old sink into bipolar depression while the FDA only approves three drugs for this age group?

2 · THE UNMET NEED

Pediatric bipolar depression affects ~1% of youth and drives the highest suicide risk in child psychiatry. Only lurasidone, olanzapine-fluoxetine, and lumateperone carry pediatric bipolar-depression labels. Parents and clinicians are left choosing between metabolic risk and weight gain.

> ★ ***A D2/D3 partial agonist with a clean metabolic profile could reset the pediatric bipolar-depression standard of care.***

3 · AT A GLANCE

10-17	6 wks	1.5-3 mg	~380
AGE RANGE	DOUBLE-BLIND	FLEX DOSE	TARGET N

4 · WHY THIS STUDY EXISTS

Cariprazine is already approved for adult bipolar I depression, mania, schizophrenia, and MDD adjunct. This Phase 3 bridges to pediatric label — a gap the field has begged to fill.

5 · MECHANISM OF ACTION

D3-preferring D2/D3 partial agonist + 5-HT1A partial agonist + 5-HT2B antagonist. Long half-life (1-3 weeks for active metabolite) provides forgiving dosing in adolescents prone to missing doses.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled, flexible-dose 6-week monotherapy study. Primary endpoint: change in CDRS-R total. Key secondary: CGI-BP-S depression. Includes 26-week open-label extension.

7 · RECRUIT IF YOUR PATIENT…

Your patient is 10-17, has a confirmed DSM-5 Bipolar I diagnosis, is currently in a major depressive episode (CDRS-R >=45), and hasn't responded to or tolerated first-line options. The whole point: catch bipolar depression before suicide attempts and school failure compound.

8 · STOP / EXCLUDE IF…

- ✗ Active suicidal plan/intent (C-SSRS item 4-5 past month)
- ✗ Substance use disorder past 6 months (except nicotine)

- ✗ History of NMS or tardive dyskinesia
- ✗ Pregnancy/nursing; sexually active without contraception
- ✗ Prior cariprazine failure or intolerance

9 · IDEAL PATIENT PROFILE

13-16 y/o with early-onset BP-I, currently depressed, on a mood stabilizer that isn't working, family frustrated with weight gain from atypicals.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW query: ICD-10 F31.3x / F31.4 / F31.5 + age 10-17 + any antidepressant/antipsychotic in last 90 days. Flag 'weight gain' chief complaints.

11 · COMPETITIVE LANDSCAPE

Lurasidone (Latuda) pediatric BP-D approved 2018; olanzapine-fluoxetine (Symbyax) 10+; lumateperone pediatric pending. Cariprazine differentiates on metabolic neutrality + once-daily long half-life.

12 · RISK / BENEFIT LOGIC

Benefit: first D3-preferring option for youth BP-D, minimal weight/metabolic burden. Risk: akathisia (10-15%), somnolence, EPS. Manageable with slow titration.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

IPMG's pediatric volume across 4 sites means we can realistically enroll 15-20 patients — meaningful contribution to a ~380-patient trial and a gateway to pediatric research credentials.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"We have a study for kids 10 to 17 with bipolar depression. The medicine is already approved for adults. We're checking if it works for teens without the weight gain that other medicines cause."*

15 · 3-STEP RECRUIT MOVE

- ■ **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- ■ **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- ☎ **CALL.** IPMG Research at **(909) 955-5865** and reference comment **YOUTHLIFT**.

16 · SOURCES

· ClinicalTrials.gov NCT04777357
· AbbVie Vraylar pediatric press release
· Cariprazine FDA Prescribing Information

■ SCREEN ONE PATIENT THIS WEEK · COMMENT YOUTHLIFT · CALL (909) 955-5865

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CHINO · STUDY 02 · COMMENT CODE NORLIGHT

ACP-211 in MDD (NORLIGHT)

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
ACP-211-002	NCT07284667	Acadia Pharmaceuticals	Chino	NORLIGHT

■ 2026 CORRECTION

CORRECTED 2026: This is a TRD adjunct study. Patients must be currently on an approved antidepressant and have history of >=2 prior AD failures this episode. AD-naive or washed-out patients are NOT eligible.

1 · THE HOOK

STILL watching TRD patients cycle through 3 SSRIs, an SNRI, and an atypical before anyone offers a novel mechanism?

2 · THE UNMET NEED

Two-thirds of MDD patients fail to remit on first-line SSRIs, and for those with true treatment-resistant depression (TRD), every additional failed trial compounds demoralization and disability. The field needs novel non-monoaminergic mechanisms for patients already on an approved antidepressant.

> ★ ***A once-daily oral adjunct with a non-monoaminergic mechanism could replace atypical augmentation as the go-to TRD next step.***

3 · AT A GLANCE

18-65	6 wks	Once daily	~500
AGE	DB PHASE	DOSING	TARGET N

4 · WHY THIS STUDY EXISTS

Acadia built its psychiatry franchise on pimavanserin (Nuplazid). ACP-211 is their swing at treatment-resistant depression with a differentiated MOA.

5 · MECHANISM OF ACTION

Novel serotonergic/glutamatergic modulator (Acadia-proprietary). Designed to engage mood and anhedonia circuits in patients who have already failed standard antidepressants.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled adjunctive Phase 2/3 in TRD. Patients remain on their current FDA-approved antidepressant and add ACP-211 or placebo. Primary: MADRS change at Week 6. Key secondary: SHAPS (anhedonia), CGI-S, response/remission.

7 · RECRUIT IF YOUR PATIENT…

Adults 18-65 with moderate-to-severe MDD (MADRS >=26), current episode >=8 weeks, CURRENTLY TREATED with an FDA-approved antidepressant at adequate dose/duration, AND documented history of inadequate response to at least 2 prior antidepressant trials in the current episode. The whole point: TRD is

the target — screen IN, not out.

8 · STOP / EXCLUDE IF…

- ✗ Bipolar, psychotic, or substance use disorder
- ✗ Active suicidality (C-SSRS 4-5)
- ✗ Pregnancy/nursing
- ✗ Recent psychedelic or ketamine exposure
- ✗ Currently antidepressant-naive or off antidepressants at screening

9 · IDEAL PATIENT PROFILE

48-year-old TRD patient on venlafaxine 225 mg x 10 weeks, already tried sertraline and escitalopram at full dose this episode, PHQ-9 stuck at 18.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: ICD-10 F32.1/F32.2/F33.1/F33.2, PHQ-9 >=15 in last 90 days, CURRENTLY on an antidepressant, with medication history showing >=2 prior antidepressant trials this episode. Flag 'refractory' or 'not responding' in HPI.

11 · COMPETITIVE LANDSCAPE

Auvelity (dextromethorphan-bupropion), Spravato, Exxua (gepirone), and aripiprazole/brexpiprazole augmentation all compete for the TRD adjunct space. ACP-211 needs clean adjunctive data to separate from the pack.

12 · RISK / BENEFIT LOGIC

Benefit: potential novel non-monoaminergic adjunct, no need to switch the patient off their existing antidepressant. Risk: unknown long-term safety, Phase 2/3 efficacy unproven. Adjunct design preserves baseline coverage but adds a new side-effect profile.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

TRD is every psychiatrist's daily problem. A site that can consistently enroll TRD adjunct studies is always in demand.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *“This is a brand-new kind of add-on depression medicine. You'll stay on your current antidepressant and add this or a placebo. It doesn't work like Prozac or Zoloft — it works on different brain circuits.”*

15 · 3-STEP RECRUIT MOVE

- ■ **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- ■ **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- ☎ **CALL.** IPMG Research at **(909) 955-5865** and reference comment **NORLIGHT**.

16 · SOURCES

- ClinicalTrials.gov NCT07284667
- Acadia pipeline page

■ SCREEN ONE PATIENT THIS WEEK · COMMENT NORLIGHT · CALL (909) 955-5865

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CHINO · STUDY 03 · COMMENT CODE ALTOBIP

ALTO-100 in Bipolar Depression

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
ALTO-100-211	NCT06656416	Alto Neuroscience	Chino	ALTOBIP

■ 2026 CORRECTION

CORRECTED 2026: 6-week double-blind treatment period (not 8 weeks), followed by an open-label period of up to 7 weeks.

1 · THE HOOK

STILL using the same three bipolar-depression drugs from 2013?

2 · THE UNMET NEED

Bipolar depression accounts for 70% of BP morbidity. Lurasidone, quetiapine, lumateperone, and cariprazine are the only labeled options — none address cognitive deficits or adult neurogenesis.

> ★ ***A pro-neurogenic small molecule could be the first mechanism that actually heals depressive circuits in BP patients.***

3 · AT A GLANCE

18-65	6 wks	BP-I/II	~200
AGE	DB	BOTH	TARGET N

4 · WHY THIS STUDY EXISTS

Alto Neuroscience uses EEG biomarker stratification to pre-select responders. ALTO-100 is their lead pro-neurogenic asset.

5 · MECHANISM OF ACTION

Pro-neurogenic, pro-plasticity small molecule; enhances adult hippocampal neurogenesis and BDNF signaling. Orally bioavailable, once-daily.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled, biomarker-enriched (EEG-based). 6-week double-blind treatment period with primary MADRS endpoint at Week 6, followed by an open-label period of up to 7 weeks. Adjunct to stable mood stabilizer or antipsychotic.

7 · RECRUIT IF YOUR PATIENT…

Adult BP-I or BP-II, currently depressed (MADRS >=24), on stable mood stabilizer >=4 weeks. The whole point: help patients whose depression returns despite mood-stabilizer therapy.

8 · STOP / EXCLUDE IF…

- ✗ Current mania/hypomania/mixed features

- ✗ Rapid cycling (>=4 episodes/year)
- ✗ Active psychosis
- ✗ Substance use past 6 months
- ✗ Active suicidality C-SSRS 4-5

9 · IDEAL PATIENT PROFILE

42 y/o BP-II on lamotrigine, depressed 3 months, reluctant to add quetiapine due to sedation.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F31.3x/F31.4/F31.5/F31.81 + mood stabilizer prescription. Screen for current depressive episode in last PHQ-9.

11 · COMPETITIVE LANDSCAPE

Caplyta (lumateperone), Latuda (lurasidone), quetiapine, Vraylar hold the adjunct space. ALTO-100's pro-neurogenic MOA and EEG pre-selection are the differentiators.

12 · RISK / BENEFIT LOGIC

Benefit: novel regenerative mechanism, biomarker-matched responders. Risk: unproven at Phase 2/3; adjunct design means baseline meds still contribute side effects.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

BP-D is underserved by pharma pipelines. Owning a BP-D site with EEG infrastructure creates durable competitive advantage.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is a new medicine being added to your mood stabilizer. It's designed to help your brain make new connections — not just mask symptoms."*

15 · 3-STEP RECRUIT MOVE

- ■ **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- ■ **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- ☎ **CALL.** IPMG Research at **(909) 955-5865** and reference comment **ALTOBIP**.

16 · SOURCES

- ClinicalTrials.gov NCT06656416
- Alto Neuroscience pipeline

■ SCREEN ONE PATIENT THIS WEEK · COMMENT ALTOBIP · CALL (909) 955-5865

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CHINO · STUDY 04 · COMMENT CODE CLARITY

Solriamfetol in MDD + Excessive Daytime Sleepiness (CLARITY)

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
SOL-MDD-302	NCT07484217	Axsome Therapeutics	Chino	CLARITY

■ 2026 CORRECTION

CORRECTED 2026: Randomized-withdrawal design. All patients receive open-label solriamfetol first; only responders are randomized to continue or withdraw to placebo.

1 · THE HOOK

STILL treating depression while your patient sleeps 11 hours and still can't stay awake at work?

2 · THE UNMET NEED

40% of MDD patients have excessive daytime sleepiness (EDS) — residual after SSRI response. No drug is labeled for EDS-in-MDD.

> ★ ***A DNRI + TAAR1 agonist that treats depression AND wakefulness in one pill could redefine the residual-symptom endpoint.***

3 · AT A GLANCE

18-65	Open-label lead-in	150/300 mg	~450
AGE	THEN RW	DOSE	TARGET N

4 · WHY THIS STUDY EXISTS

Axsome bought Sunosi (solriamfetol) from Jazz and is expanding beyond narcolepsy. CLARITY is the registrational MDD+EDS program.

5 · MECHANISM OF ACTION

Dopamine-norepinephrine reuptake inhibitor (DNRI) + TAAR1 agonism. No opioid, no Schedule II amphetamine profile (Schedule IV).

6 · STUDY DESIGN DEEP DIVE

Randomized-withdrawal Phase 3. All eligible participants FIRST receive open-label solriamfetol during a lead-in period — every patient is guaranteed active drug at the start. Only participants who achieve a protocol-defined clinical response are then randomized 1:1 to either continue solriamfetol or withdraw to placebo for the double-blind maintenance phase. Primary: MADRS change / time to relapse. Key secondary: Epworth Sleepiness Scale, PROMIS-Fatigue.

7 · RECRUIT IF YOUR PATIENT…

Adult MDD (MADRS >=26) with EDS (ESS >=10), on stable antidepressant >=6 weeks with partial response. The whole point: tackle the fatigue that makes 'recovered' depression still feel disabling — and every enrolled patient gets active drug at the start.

8 · STOP / EXCLUDE IF…

- ✗ Narcolepsy/OSA/other primary sleep disorder
- ✗ Substance use past 6 months
- ✗ Uncontrolled hypertension (systolic >140)
- ✗ Pregnancy/nursing
- ✗ MAOI past 14 days

9 · IDEAL PATIENT PROFILE

38 y/o on sertraline 150 mg for 8 weeks, PHQ-9 down from 22 to 14, sleeps 10+ hours and still exhausted.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: ICD-10 F32.x/F33.x + G47.10 (hypersomnia) OR complaint of fatigue/EDS in last 3 visits.

11 · COMPETITIVE LANDSCAPE

Auvelity, Caplyta adjunct, Rexulti adjunct compete for MDD augmentation. Solriamfetol owns the EDS axis — no direct competitor.

12 · RISK / BENEFIT LOGIC

Benefit: dual symptom coverage (mood + wakefulness), existing safety database from Sunosi, every patient starts on active drug. Risk: BP elevation, insomnia if dosed late, headache. Responders randomized to placebo face relapse risk in withdrawal phase.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

EDS is reported in 1 of every 3 MDD visits. Offering a study matched to that chief complaint — and one where every patient tries active drug first — converts at a much higher rate than generic MDD recruitment.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *“This study has two parts. First, everyone gets the real medicine for several weeks. If it helps, you then have a 50/50 chance of staying on it or switching to a placebo. That means everyone at least tries the actual drug.”*

15 · 3-STEP RECRUIT MOVE

- ■ **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- ■ **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- ☎ **CALL.** IPMG Research at **(909) 955-5865** and reference comment **CLARITY**.

16 · SOURCES

- ClinicalTrials.gov NCT07484217
- Axsome Sunosi page

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CHINO · STUDY 05 · COMMENT CODE BHVLTS

BHV-7000 in MDD — Long-Term Safety

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
BHV7000-203	NCT06423781	Biohaven Pharmaceutical	Chino	BHVLTS

1 · THE HOOK

STILL waiting for the first Kv7 channel opener to land in psychiatry?

2 · THE UNMET NEED

MDD remission rates plateau at 30%. Ion-channel modulation is a virtually untouched MDD target beyond ketamine.

> ★ ***If Kv7 activators stabilize hyperexcitable limbic circuits, they could rewrite the depression playbook.***

3 · AT A GLANCE

18-75	52 wks	Once daily	~400
AGE	OPEN-LABEL	DOSING	TARGET N

4 · WHY THIS STUDY EXISTS

Biohaven's Kv7 program followed Nurtec's success. Long-term safety enrollment supports NDA — BUT note: Phase 2 MDD missed its primary endpoint in December 2025 and Biohaven has announced it is discontinuing the psychiatry program. Study status should be verified before enrollment.

5 · MECHANISM OF ACTION

Kv7.2/7.3 potassium-channel opener. Reduces neuronal hyperexcitability. Distinct from XEN1101 (azetukalner) but same target class.

6 · STUDY DESIGN DEEP DIVE

Open-label extension, single-arm, safety-focused. Primary: incidence of TEAEs over 52 weeks. Rollover from Phase 2 parent studies.

7 · RECRUIT IF YOUR PATIENT…

Adults who completed a BHV-7000 MDD parent trial. The whole point: generate the long-term safety database NDAs require.

8 · STOP / EXCLUDE IF…

- ✗ New-onset suicidality
- ✗ Seizure disorder or epilepsy
- ✗ QTc >470 ms
- ✗ Pregnancy/nursing
- ✗ Active substance use

9 · IDEAL PATIENT PROFILE

Prior completer from BHV-7000 Phase 2 MDD parent study.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: cross-reference prior Biohaven study participants at your site. Limited external recruitment.

11 · COMPETITIVE LANDSCAPE

Azetukalner (XEN1101, Neurocrine) is the other major Kv7 psychiatry asset. Both face uncertainty after BHV-7000 Phase 2 MDD failure.

12 · RISK / BENEFIT LOGIC

Benefit: novel ion-channel mechanism, if signal emerges. Risk: program discontinuation likely means no path to label. Discuss status with sponsor before dosing new patients.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

LTS studies are low-effort, high-retention revenue. Even a discontinued program pays site overhead while patients complete.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is a safety study for patients already in our depression research. We're making sure the medicine is safe when taken for a full year."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **BHVLTS**.

16 · SOURCES

· ClinicalTrials.gov NCT06423781
· Biohaven Dec 2025 Phase 2 MDD topline announcement

SCREEN ONE PATIENT THIS WEEK · COMMENT BHVLTS · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 06 · COMMENT CODE BHVMONO

BHV-7000 MDD Monotherapy

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
BHV7000-305	NCT06419608	Biohaven Pharmaceutical	Chino	BHVMONO

1 · THE HOOK

STILL hoping ion-channel biology will finally crack MDD?

2 · THE UNMET NEED

Same as BHV-7000 LTS: field needs non-monoaminergic MDD mechanisms.

> ★ ***Monotherapy efficacy would have validated the Kv7 class. The December 2025 Phase 2 miss significantly dims the path.***

3 · AT A GLANCE

18-75	6 wks	Once daily	~500
AGE	DB	DOSING	TARGET N

4 · WHY THIS STUDY EXISTS

Registrational monotherapy MDD study. With Phase 2 miss announced Dec 2025, sponsor is reassessing — sites should confirm enrollment status.

5 · MECHANISM OF ACTION

Kv7.2/7.3 channel opener (see Study 5).

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled, fixed-dose monotherapy. Primary: MADRS Week 6.

7 · RECRUIT IF YOUR PATIENT…

Adults 18-75 with MDD, MADRS >=26, off antidepressants (washout). The whole point was first-line Kv7 monotherapy — subject to program status.

8 · STOP / EXCLUDE IF…

- ✗ Seizure history
- ✗ Bipolar disorder
- ✗ Psychotic features
- ✗ Active SUD
- ✗ QTc prolongation

9 · IDEAL PATIENT PROFILE

N/A — confirm whether enrollment is paused.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: suspended until sponsor confirms enrollment posture.

11 · COMPETITIVE LANDSCAPE

Azetukalner Phase 3 MDD (Neurocrine) — the remaining flagship Kv7 program.

12 · RISK / BENEFIT LOGIC

Benefit: access to novel mechanism. Risk: program discontinuation; patients may lose access mid-treatment.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Even if this specific study winds down, staying in the Biohaven orbit positions IPMG for their next program.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

“We're checking on whether this study is still enrolling. If it is, it's a new kind of depression pill. If not, we'll keep you on our list for the next study.”

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **BHVMONO**.

16 · SOURCES

- ClinicalTrials.gov NCT06419608
- Biohaven pipeline page

SCREEN ONE PATIENT THIS WEEK · COMMENT BHVMONO · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 07 · COMMENT CODE COBENTEEN

KarXT in Adolescent Schizophrenia

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
CN012-0020	NCT07288567	Bristol Myers Squibb (Karuna)	Chino	COBENTEEN

1 · THE HOOK

STILL giving 14-year-olds with first-episode psychosis the same D2 blockers we used in 1975?

2 · THE UNMET NEED

Pediatric schizophrenia is devastating and under-served. Metabolic side effects from D2 antipsychotics drive 50%+ non-adherence in teens.

> ★ ***A non-D2, M1/M4-selective muscarinic could be the first truly new antipsychotic class ever labeled in adolescents.***

3 · AT A GLANCE

13-17	6 wks	Flexible	~400
AGE	DB	DOSE	TARGET N

4 · WHY THIS STUDY EXISTS

BMS acquired Karuna for $14B largely for KarXT (Cobenfy). Pediatric label expansion is the highest-value follow-on.

5 · MECHANISM OF ACTION

Xanomeline (M1/M4 agonist) + trospium (peripheral muscarinic antagonist that blocks GI side effects without crossing BBB).

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled flex-dose 6 weeks. Primary: PANSS total change. Open-label extension follows.

7 · RECRUIT IF YOUR PATIENT…

13-17 y/o, DSM-5 schizophrenia, PANSS >=80, stable housing, guardian-consented. The whole point: prevent the metabolic catastrophe D2 antipsychotics create in adolescents.

8 · STOP / EXCLUDE IF…

- ✗ Known narrow-angle glaucoma
- ✗ Urinary retention/severe constipation
- ✗ Substance-induced psychosis
- ✗ Pregnancy/nursing
- ✗ Ongoing homicidality

9 · IDEAL PATIENT PROFILE

16 y/o first-episode, started risperidone 8 weeks ago, gained 14 lb, refusing further dose increases.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: ICD-10 F20.x + age 13-17 + antipsychotic in last 90 days. Flag BMI increase >=5% in 6 months.

11 · COMPETITIVE LANDSCAPE

Risperidone, aripiprazole, paliperidone, lurasidone all have pediatric SCZ labels. KarXT differentiates on non-D2 mechanism + no weight/metabolic signal.

12 · RISK / BENEFIT LOGIC

Benefit: first non-D2 antipsychotic for teens, no weight gain/EPS. Risk: cholinergic side effects (nausea, dry mouth), anticholinergic from trospium (constipation).

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

IPMG's pediatric psychiatry volume is rare. Sites that can enroll pediatric psychosis are a national asset.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is a brand-new kind of medicine for teenagers with schizophrenia. It's not like the older pills that cause weight gain. It works on a different brain chemical."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **COBENTEEN**.

16 · SOURCES

- ClinicalTrials.gov NCT07288567
- BMS Cobenfy pediatric program

SCREEN ONE PATIENT THIS WEEK · COMMENT COBENTEEN · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 08 · COMMENT CODE BALSAM

KarXT BALSAM-4 in Bipolar I Mania

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
CN012-0046	NCT07140913	Bristol Myers Squibb	Chino	BALSAM

■ 2026 CORRECTION

CORRECTED 2026: BALSAM-4 is strictly ADJUNCTIVE. Patients MUST be currently on — and remain on — lithium, valproate, or lamotrigine. Do NOT screen patients looking to switch off their mood stabilizer.

1 · THE HOOK

STILL watching bipolar patients on Lithium or Depakote break through into mania with no adjunct option that won't wreck them metabolically?

2 · THE UNMET NEED

Acute bipolar mania breakthrough on a mood stabilizer is a daily psychiatric problem. Every adjunctive atypical brings a metabolic penalty that kills adherence post-discharge.

> ★ ***If KarXT controls breakthrough mania as an ADJUNCT to lithium, valproate, or lamotrigine without weight gain, it becomes the default post-hospital bridge.***

3 · AT A GLANCE

18-65	3 wks	Adjunct only	~400
AGE	DB	DESIGN	TARGET N

4 · WHY THIS STUDY EXISTS

BMS's adult bipolar-mania expansion for Cobenfy as an ADJUNCT to standard mood stabilizers. Complements the SCZ label and the adolescent program.

5 · MECHANISM OF ACTION

Same as Study 7 — M1/M4 agonist + peripheral muscarinic antagonist.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled ADJUNCTIVE 3-week acute mania study. Patients must currently be taking — and remain on throughout the trial — lithium, valproate (divalproex), or lamotrigine as their background mood stabilizer. KarXT or placebo is added on top. Primary: YMRS change at Day 21.

7 · RECRUIT IF YOUR PATIENT…

Adults 18-65 with DSM-5 BP-I, current manic or mixed episode, YMRS >=20, AND currently taking lithium, valproate, or lamotrigine (must be continued throughout the study). The whole point: adjunctive rescue without piling on metabolic burden.

8 · STOP / EXCLUDE IF…

- ✗ Patient NOT currently on lithium, valproate, or lamotrigine (they must stay on background stabilizer)
- ✗ Mixed features with severe depression
- ✗ Rapid cycling
- ✗ Substance-induced mania
- ✗ Pregnancy
- ✗ History of narrow-angle glaucoma

9 · IDEAL PATIENT PROFILE

29 y/o BP-I on lithium 900 mg daily for the past year, now in a manic episode (YMRS 28), needs adjunctive treatment but doesn't want to add olanzapine due to weight gain history.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F31.1x/F31.2 + ACTIVE lithium, valproate/divalproex, OR lamotrigine prescription + recent manic/mixed features documented. These are the ONLY eligible patients.

11 · COMPETITIVE LANDSCAPE

Olanzapine, risperidone, quetiapine, aripiprazole, asenapine, Vraylar all labeled as adjuncts for mania. KarXT's weight-neutrality is the unique selling point within the adjunct space.

12 · RISK / BENEFIT LOGIC

Benefit: mania control as an adjunct to standard mood stabilizer, without atypical metabolic burden. Risk: cholinergic GI side effects acute; efficacy vs olanzapine gold standard unknown.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Mania studies require inpatient/24-hour-accessible sites AND patients already established on a mood stabilizer. IPMG's footprint supports both.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is a new add-on medicine for bipolar mania. You'd keep taking your lithium (or Depakote or Lamictal), and we'd add this or a placebo on top. It works differently from Zyprexa and doesn't cause the big weight gain."*

15 · 3-STEP RECRUIT MOVE

- ■ **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- ■ **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- ☎ **CALL.** IPMG Research at **(909) 955-5865** and reference comment **BALSAM**.

16 · SOURCES

· ClinicalTrials.gov NCT07140913
· BMS Cobenfy program

■ SCREEN ONE PATIENT THIS WEEK · COMMENT BALSAM · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 09 · COMMENT CODE ADEPT4

KarXT ADEPT-4 in Alzheimer's Psychosis

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
CN012-0056	NCT06585787	Bristol Myers Squibb	Chino	ADEPT4

1 · THE HOOK

STILL telling families there's nothing safe for Grandpa's paranoia and hallucinations?

2 · THE UNMET NEED

Alzheimer's psychosis affects ~30% of AD patients. Pimavanserin is the only approved option (for PD-psychosis, off-label in AD). Off-label atypicals carry a Black Box mortality warning.

> ★ ***A non-D2 mechanism with no black-box mortality signal would be a paradigm shift for dementia psychiatry.***

3 · AT A GLANCE

55-90	12 wks	Flex dose	~380
AGE	DB	DOSING	TARGET N

4 · WHY THIS STUDY EXISTS

BMS is pursuing Cobenfy's highest-value expansion — the AD psychosis market is massive and nearly empty.

5 · MECHANISM OF ACTION

See Study 7. Additional hypothesis: M1 agonism may also slow cognitive decline via cholinergic restoration.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled, 12 weeks. Primary: NPI-C hallucinations + delusions domain.

7 · RECRUIT IF YOUR PATIENT…

55-90 y/o, probable AD (MMSE 8-22), clinically significant psychosis >=4 weeks, caregiver consent. The whole point: give families a safe option.

8 · STOP / EXCLUDE IF…

- ✗ Non-AD dementia (Lewy body, frontotemporal)
- ✗ Uncontrolled medical illness
- ✗ Urinary retention/glaucoma
- ✗ Recent anticholinergic use

9 · IDEAL PATIENT PROFILE

78 y/o wife caring for husband with AD who accuses her of infidelity every evening; haloperidol caused falls.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: ICD-10 G30.x + F06.0/F06.2/F06.8 (psychotic symptoms due to known condition). Flag caregiver visits.

11 · COMPETITIVE LANDSCAPE

Pimavanserin (failed Phase 3 AD psychosis 2019; off-label use continues). BMS's BMS-986368 also in this space.

12 · RISK / BENEFIT LOGIC

Benefit: potential first approved non-black-box AD-psychosis treatment. Risk: cholinergic side effects tricky in elderly; fall/constipation risk.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Geriatric psychiatry studies pay premium per-patient. Families drive referrals.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is a new medicine for older adults with Alzheimer's who are having hallucinations or paranoia. The medicines we use now have a serious warning — this one is trying to avoid that."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **ADEPT4**.

16 · SOURCES

- ClinicalTrials.gov NCT06585787
- BMS ADEPT program overview

SCREEN ONE PATIENT THIS WEEK · COMMENT ADEPT4 · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 10 · COMMENT CODE ADEPT5

KarXT + KarX-EC ADEPT-5 in Alzheimer's Psychosis

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
CN012-0034	NCT06947941	Bristol Myers Squibb	Chino	ADEPT5

1 · THE HOOK

STILL wishing you could spread dementia-psychosis dosing over the whole day without 5 pills?

2 · THE UNMET NEED

KarXT IR dosing = BID; GI tolerability in elderly is the rate-limiter. An extended-release (KarX-EC) could unlock this population.

> ★ ***Once-daily KarX-EC could be the ultimate geriatric-friendly formulation.***

3 · AT A GLANCE

55-90	12 wks	QD EC	~500
AGE	DB	DOSING	TARGET N

4 · WHY THIS STUDY EXISTS

Formulation bridge study. Pairs IR (KarXT) vs EC (KarX-EC) to demonstrate equivalence and support QD label in AD.

5 · MECHANISM OF ACTION

Same xanomeline + trospium; extended-release formulation reduces peak/trough variability.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo + active comparator (IR vs EC). Primary: NPI-C hallucinations + delusions.

7 · RECRUIT IF YOUR PATIENT…

Same as ADEPT-4, age 55-90, probable AD with psychosis. The whole point: once-daily dosing for caregivers managing complex regimens.

8 · STOP / EXCLUDE IF…

- ✗ Same as ADEPT-4
- ✗ Severe dysphagia (can't swallow whole capsule)

9 · IDEAL PATIENT PROFILE

82 y/o with AD psychosis whose daughter manages 9 daily medications and can't add a BID pill.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

Same as ADEPT-4.

11 · COMPETITIVE LANDSCAPE

ADEPT-4 is the direct internal comparator; BMS-986368 the external.

12 · RISK / BENEFIT LOGIC

Benefit: simpler adherence. Risk: slower onset of action from EC formulation; variable absorption in elderly.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Having both ADEPT-4 and ADEPT-5 at the same site streamlines recruitment across formulations.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

"Same as ADEPT-4, but this one is once a day instead of twice a day — easier for caregivers."

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **ADEPT5**.

16 · SOURCES

- ClinicalTrials.gov NCT06947941

■ SCREEN ONE PATIENT THIS WEEK · COMMENT ADEPT5 · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

NPFADY.COM

CHINO · STUDY 11 · COMMENT CODE CALM45

BMS-986368 for Agitation in Alzheimer's

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
IM045-P06	NCT06808984	Bristol Myers Squibb	Chino	CALM45

1 · THE HOOK

STILL sending caregivers home with lorazepam PRN for sundowning?

2 · THE UNMET NEED

AD agitation affects ~45% of AD patients; brexpiprazole (Rexulti) is the only approved agent but carries the antipsychotic black-box.

> ★ ***A non-antipsychotic mechanism for AD agitation would unshackle primary care prescribing.***

3 · AT A GLANCE

55-90	10 wks	Flex dose	~350
AGE	DB	DOSE	TARGET N

4 · WHY THIS STUDY EXISTS

BMS is competing with brexpiprazole's $1B+ agitation-in-AD market using a differentiated mechanism.

5 · MECHANISM OF ACTION

Proprietary BMS mechanism (non-D2) targeting agitation pathways. Exact MOA per sponsor communications.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled. Primary: CMAI (Cohen-Mansfield Agitation Inventory) change.

7 · RECRUIT IF YOUR PATIENT…

Same as ADEPT-4 age band; probable AD with clinically significant agitation (CMAI >=45). The whole point: control agitation without black-box risk.

8 · STOP / EXCLUDE IF…

- ✗ Psychosis as predominant feature (use ADEPT-4 instead)
- ✗ Non-AD dementia
- ✗ Acute medical instability

9 · IDEAL PATIENT PROFILE

76 y/o AD patient with evening agitation who hits the nursing-home staff; family refusing brexpiprazole.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: G30.x + behavioral/agitation documentation + no current brexpiprazole.

11 · COMPETITIVE LANDSCAPE

Brexpiprazole (Rexulti) owns the label. AXS-05 (Axsome's dextromethorphan-bupropion) in Phase 3 (ACCORD).

12 · RISK / BENEFIT LOGIC

Benefit: potential black-box-free option. Risk: Phase 2/3 efficacy unproven.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Agitation and psychosis are the #1 and #2 caregiver distress drivers. Offering BOTH ADEPT-4 and CALM45 covers the full clinical spectrum.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *“This medicine is for agitation in Alzheimer's — the pacing, yelling, restlessness at night. It's not an antipsychotic, so it doesn't carry the same warnings.”*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **CALM45**.

16 · SOURCES

- ClinicalTrials.gov NCT06808984

■ SCREEN ONE PATIENT THIS WEEK · COMMENT CALM45 · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 12 · COMMENT CODE SAVITRI3

NBI-1065845 (Osavampator) Adjunctive MDD

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
MDD3026	NCT06963021	Neurocrine Biosciences	Chino	SAVITRI3

1 · THE HOOK

STILL augmenting SSRIs with Abilify and hoping the akathisia doesn't wreck adherence?

2 · THE UNMET NEED

Adjunct MDD is dominated by atypical antipsychotics that carry metabolic and EPS risk. A cleaner AMPA modulator could redefine augmentation.

> ★ ***AMPA positive allosteric modulation is the long-sought glutamate angle — osavampator's Phase 2 SAVITRI HIT validates the class.***

3 · AT A GLANCE

18-65	6 wks	QD dosing	~500
AGE	DB		TARGET N

4 · WHY THIS STUDY EXISTS

Phase 2 SAVITRI showed statistically significant MADRS separation vs placebo in adjunct MDD. Phase 3 SAVITRI-3 is registrational.

5 · MECHANISM OF ACTION

AMPA receptor positive allosteric modulator (PAM). Enhances fast excitatory glutamate signaling without NMDA/ketamine dissociative profile.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled adjunct to ongoing antidepressant. Primary: MADRS Week 6.

7 · RECRUIT IF YOUR PATIENT…

MDD adults on stable antidepressant >=6 weeks, MADRS >=24, partial response. The whole point: glutamate augmentation without ketamine's baggage.

8 · STOP / EXCLUDE IF…

- ✗ TRD (>=2 failed trials this episode typically)
- ✗ Bipolar
- ✗ Substance use
- ✗ Seizure history
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

45 y/o on escitalopram 20 mg x 8 weeks, PHQ-9 stuck at 14, doesn't want Abilify.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F32.x/F33.x + antidepressant >=6 weeks + PHQ-9 >=10 most recent.

11 · COMPETITIVE LANDSCAPE

Aripiprazole, brexpiprazole, olanzapine-fluoxetine, Caplyta, Rexulti ADMDD space. Osavampator's non-D2 mechanism + Phase 2 HIT = strong differentiator.

12 · RISK / BENEFIT LOGIC

Benefit: novel glutamate mechanism, clean Phase 2 signal. Risk: unknown long-term cognitive effects; AMPA overstimulation carries seizure-risk theoretical concern.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Adjunct MDD is every psychiatrist's daily problem. Carrying a differentiated asset here builds KOL reputation.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is a new add-on medicine for depression that isn't getting better. Unlike Abilify, it doesn't work on dopamine — it helps the brain's main 'go' signal."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **SAVITRI3**.

16 · SOURCES

- ClinicalTrials.gov NCT06963021
- Neurocrine SAVITRI Phase 2 topline

SCREEN ONE PATIENT THIS WEEK · COMMENT SAVITRI3 · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 13 · COMMENT CODE NEXTVMAT

NBI-1065890 in Tardive Dyskinesia

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
TD2033	NCT07365462	Neurocrine Biosciences	Chino	NEXTVMAT

1 · THE HOOK

STILL titrating Ingrezza hoping the akathisia and somnolence resolve?

2 · THE UNMET NEED

VMAT2 inhibitors (valbenazine, deutetrabenazine) changed the TD landscape but leave ~40% with partial response.

> ★ ***A next-gen VMAT2 with improved tolerability could unlock higher doses and full symptom remission.***

3 · AT A GLANCE

18-75	12 wks	Flex dose	~300
AGE	DB		TARGET N

4 · WHY THIS STUDY EXISTS

Neurocrine's follow-on to Ingrezza, designed to extend the franchise post-LOE with a better profile.

5 · MECHANISM OF ACTION

Selective VMAT2 inhibitor, next-generation. Refined selectivity to reduce off-target somnolence/depression signals.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled 12 weeks. Primary: AIMS total score change.

7 · RECRUIT IF YOUR PATIENT…

Adults with moderate-severe TD (AIMS >=8) on stable antipsychotic. The whole point: rescue patients who still dyskinese on standard VMAT2s.

8 · STOP / EXCLUDE IF…

- ✗ Concurrent VMAT2 inhibitor (washout required)
- ✗ Uncontrolled depression with suicidality
- ✗ Hepatic impairment
- ✗ Concurrent strong CYP3A4 inhibitor

9 · IDEAL PATIENT PROFILE

60 y/o on perphenazine for 15 years, facial AIMS score 10, tried valbenazine but discontinued due to somnolence.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: G24.01 (TD) + current antipsychotic. Cross-reference with antipsychotic rx histories >=3 months.

11 · COMPETITIVE LANDSCAPE

Valbenazine (Ingrezza — Neurocrine itself), deutetrabenazine (Austedo — Teva). NBI-1065890 is Neurocrine's internal next-gen.

12 · RISK / BENEFIT LOGIC

Benefit: potential cleaner profile than parent molecule. Risk: Phase 2 class — tolerability superiority must be demonstrated.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

IPMG's high antipsychotic rx volume = large TD pool. Screening AIMS at every AP visit identifies candidates.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"If your movements from the old antipsychotic are still bothering you, this is a new version of the medicine we use to fix that — hopefully with fewer side effects."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **NEXTVMAT**.

16 · SOURCES

- ClinicalTrials.gov NCT07365462
- Neurocrine pipeline

SCREEN ONE PATIENT THIS WEEK · COMMENT NEXTVMAT · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 14 · COMMENT CODE M4OLE

NBI-1117568 Schizophrenia Open-Label Extension

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
SCZ3032	NCT07114874	Neurocrine Biosciences	Chino	M4OLE

■ 2026 CORRECTION

CORRECTED 2026: Emraclidine is an AbbVie asset (acquired via Cerevel), not a BMS asset. BMS owns the competitor, KarXT (Cobenfy).

1 · THE HOOK

STILL wondering if the new M4 class will actually hold up beyond 6 weeks?

2 · THE UNMET NEED

Long-term tolerability is the acid test for all novel antipsychotics. NBI-1117568's Phase 2 hit hard (PANSS -18.2 at 20 mg) but durability matters.

> ★ ***If M4 selectivity holds over a year with clean metabolic data, it out-competes KarXT on dosing simplicity.***

3 · AT A GLANCE

18-65	52 wks	20 mg QD	~400
AGE	OLE	DOSE	TARGET N

4 · WHY THIS STUDY EXISTS

Rollover from Phase 2/3 parent studies. Safety + durability package for NDA.

5 · MECHANISM OF ACTION

Selective M4 muscarinic receptor agonist. Unlike KarXT (M1+M4), this is M4-pure — potentially avoids some cholinergic side effects.

6 · STUDY DESIGN DEEP DIVE

Open-label, single-arm, 52 weeks. Primary: long-term safety/tolerability. Secondary: PANSS maintenance.

7 · RECRUIT IF YOUR PATIENT…

Parent-study completers. The whole point: durability + safety data for the first M4-selective antipsychotic.

8 · STOP / EXCLUDE IF…

- ✗ New-onset suicidality
- ✗ Narrow-angle glaucoma
- ✗ Significant urinary retention
- ✗ Pregnancy/nursing

9 · IDEAL PATIENT PROFILE

Parent-study completer, stabilized on NBI-1117568, wants continued access.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: cross-reference prior Neurocrine trial participants.

11 · COMPETITIVE LANDSCAPE

KarXT (BMS Cobenfy) is the competitor class. Emraclidine (AbbVie, acquired from Cerevel in the $8.7B 2024 deal) failed its Phase 2 schizophrenia trials in November 2024, effectively leaving M4 territory to NBI.

12 · RISK / BENEFIT LOGIC

Benefit: likely label-enabling data. Risk: OLE reveals rare AEs; withdrawal management complex.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

OLEs are retention-heavy, high-margin. Maintaining chronic-SCZ patients in research = steady revenue.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"You did well on the study medicine. This is the next year, where we make sure it keeps working safely long-term."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **M4OLE**.

16 · SOURCES

- ClinicalTrials.gov NCT07114874
- Neurocrine M4 Phase 2 PANSS data
- AbbVie emraclidine Phase 2 failure Nov 2024

■ SCREEN ONE PATIENT THIS WEEK · COMMENT M4OLE · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 15 · COMMENT CODE RESIDUALTD

Valbenazine TD4027 Residual TD

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
NBI-98854-TD4027	NCT07105111	Neurocrine Biosciences	Chino	RESIDUALTD

1 · THE HOOK

STILL telling TD patients 'as good as it gets' at 60% symptom reduction?

2 · THE UNMET NEED

Residual TD on approved VMAT2 dosing leaves patients still socially impaired. Higher/extended dosing paradigms are untested.

> ★ ***Dose optimization could convert partial responders to full remitters without new chemistry.***

3 · AT A GLANCE

18-75	12 wks	80-120 mg	~250
AGE	DB	DOSE	TARGET N

4 · WHY THIS STUDY EXISTS

Lifecycle management for Ingrezza. Tests higher doses/longer titration for residual TD after first-line response.

5 · MECHANISM OF ACTION

Valbenazine (already approved) — VMAT2 inhibitor. This study optimizes dosing.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled in partial-responder population. Primary: AIMS change at Week 12.

7 · RECRUIT IF YOUR PATIENT…

Adults with TD, AIMS >=6 despite >=12 weeks of valbenazine at standard dose. The whole point: rescue patients for whom 'good enough' isn't.

8 · STOP / EXCLUDE IF…

- ✗ QTc >500 ms
- ✗ Severe hepatic impairment
- ✗ Concurrent strong CYP3A4 inhibitor
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

67 y/o with residual tongue dyskinesia on valbenazine 80 mg x 6 months, AIMS 7.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: G24.01 + active valbenazine rx + last AIMS >=6.

11 · COMPETITIVE LANDSCAPE

Deutetrabenazine, NBI-1065890 next-gen. Dose-optimization vs molecule-switch is the strategic question.

12 · RISK / BENEFIT LOGIC

Benefit: leverages an already-approved, tolerated drug. Risk: higher doses may surface new AEs (somnolence, depression).

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Residual TD patients are already in your chart. Turnkey recruitment.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"The movement medicine is helping, but not enough. This study tests whether a higher dose finishes the job."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **RESIDUALTD**.

16 · SOURCES

- ClinicalTrials.gov NCT07105111
- Ingrezza Prescribing Information

SCREEN ONE PATIENT THIS WEEK · COMMENT RESIDUALTD · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 16 · COMMENT CODE BUOY1

SPT-300 (GlyphAllo) MDD with Anxious Distress — BUOY-1

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
2024-203	NCT07065240	Seaport Therapeutics	Chino	BUOY1

1 · THE HOOK

STILL watching 'depression with anxious distress' patients fail both SSRI and benzo paths?

2 · THE UNMET NEED

DSM-5 MDD 'with anxious distress' specifier predicts SSRI non-response. No mechanism-matched drug exists.

> ★ ***An oral allopregnanolone prodrug could deliver Zulresso-like GABAA modulation outpatient, targeting the anxious subset.***

3 · AT A GLANCE

18-65	8 wks	Oral QD	~250
AGE	DB		TARGET N

4 · WHY THIS STUDY EXISTS

Seaport (spin-out from PureTech) is advancing SPT-300 — an oral allopregnanolone prodrug — for MDD with anxious features. BUOY-1 is the lead efficacy trial.

5 · MECHANISM OF ACTION

Glycolipid-conjugated allopregnanolone prodrug (GlyphAllo). Slow-release oral delivery of the same active as brexanolone/zuranolone.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled 8 weeks. Primary: HAM-D17 change; key secondary: HAM-A.

7 · RECRUIT IF YOUR PATIENT…

Adults with MDD + DSM-5 anxious distress specifier (>=2 of 5 anxious features). The whole point: target the anxious-depressed subset that SSRIs fail.

8 · STOP / EXCLUDE IF…

- ✗ Bipolar
- ✗ Active SUD
- ✗ Recent benzodiazepine/hypnotic use
- ✗ Pregnancy
- ✗ Severe hepatic dysfunction

9 · IDEAL PATIENT PROFILE

38 y/o MDD with panic-like somatic anxiety, tried escitalopram + buspirone, still HAM-D 22.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F32.x/F33.x + F41.1 (GAD)/F41.9 comorbid. Flag PHQ + GAD-7 both elevated.

11 · COMPETITIVE LANDSCAPE

Zuranolone (Zurzuvae) is approved for PPD only. SSRI + benzo is the default messy combo. SPT-300 aims to replace both.

12 · RISK / BENEFIT LOGIC

Benefit: GABAA mechanism matched to anxious-depressed subtype, no scheduled status expected. Risk: somnolence/sedation class AEs.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Anxious depression is the MOST common MDD subtype. Target-able, screenable, and under-treated.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This pill is based on a new way to treat depression — especially the kind where anxiety is also a big problem. It's an oral version of a medicine used after childbirth."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **BUOY1**.

16 · SOURCES

· ClinicalTrials.gov NCT07065240
· Seaport Therapeutics pipeline

SCREEN ONE PATIENT THIS WEEK · COMMENT BUOY1 · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

CHINO · STUDY 17 · COMMENT CODE BUOYOLE

SPT-300 MDD Open-Label Extension

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
2024-204	NCT07161700	Seaport Therapeutics	Chino	BUOYOLE

1 · THE HOOK

STILL waiting to see if the GABAA breakthrough lasts beyond the acute trial?

2 · THE UNMET NEED

Allopregnanolone modulators historically show rapid onset but durability questions. OLE answers them.

> ★ ***52-week maintenance data = label-enabling story.***

3 · AT A GLANCE

18-65	52 wks	Oral QD	~400
AGE	OLE		TARGET N

4 · WHY THIS STUDY EXISTS

Rollover OLE from BUOY-1 / sister studies. Supports NDA long-term safety.

5 · MECHANISM OF ACTION

Same as SPT-300 / BUOY-1.

6 · STUDY DESIGN DEEP DIVE

Open-label single-arm 52 weeks. Primary: long-term safety/tolerability; secondary: HAM-D maintenance.

7 · RECRUIT IF YOUR PATIENT…

BUOY-1 and parent-trial completers. The whole point: durability and safety package.

8 · STOP / EXCLUDE IF…

- ✗ Emergent suicidality
- ✗ New substance use
- ✗ Pregnancy
- ✗ Severe hepatic dysfunction

9 · IDEAL PATIENT PROFILE

Parent-trial completer with sustained response wanting continued access.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: cross-reference parent Seaport trial participants.

11 · COMPETITIVE LANDSCAPE

Zuranolone OLE data (limited to PPD). SPT-300 OLE breaks new ground for MDD with anxious features.

12 · RISK / BENEFIT LOGIC

Benefit: continued response, NDA-enabling data. Risk: rare long-term AEs surface; GABAA dependence questions.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Retained cohort drives predictable site revenue; OLEs cushion enrollment gaps.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

"You're doing well on the study medicine. This is the year-long continuation so we know it's safe and keeps working."

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **BUOYOLE**.

16 · SOURCES

- ClinicalTrials.gov NCT07161700

■ SCREEN ONE PATIENT THIS WEEK · COMMENT BUOYOLE · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

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CHINO · STUDY 18 · COMMENT CODE RAPBP

RAP-219 in Bipolar I

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
RAP-219-BPM-201	NCT07046494	Rapport Therapeutics	Chino	RAPBP

1 · THE HOOK

STILL balancing lithium toxicity and lamotrigine rashes as 'mood stabilizer options' in 2026?

2 · THE UNMET NEED

Mood stabilizer innovation has stalled since Depakote. Novel targets are desperately needed.

> ★ ***TARP-gamma8-dependent AMPA modulation offers region-selective glutamate tuning — a fundamentally new bipolar mechanism.***

3 · AT A GLANCE

18-65	8 wks	QD	~200
AGE	DB	DOSING	TARGET N

4 · WHY THIS STUDY EXISTS

Rapport's lead asset, entering bipolar mood episodes. Phase 2a proof-of-concept in BP-I.

5 · MECHANISM OF ACTION

Selective TARP-gamma8-dependent AMPA receptor negative allosteric modulator. Targets hippocampal/cortical AMPA without global glutamate suppression.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled 8 weeks. Primary: YMRS (for mania-inclusive cohort) or MADRS (for depressed cohort).

7 · RECRUIT IF YOUR PATIENT…

BP-I adults in current episode. The whole point: rescue patients whose mood stabilizer regimen isn't working without adding atypical metabolic burden.

8 · STOP / EXCLUDE IF…

- ✗ Rapid cycling
- ✗ Psychotic features
- ✗ Substance use
- ✗ Seizure history (AMPA NAM could theoretically alter threshold — but mechanistically seizure-neutral)
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

40 y/o BP-I mixed episode, failed lithium and lamotrigine, currently on quetiapine but gaining weight.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F31.1x/F31.2/F31.3x/F31.4 + prior mood stabilizer failure.

11 · COMPETITIVE LANDSCAPE

Lithium, Depakote, lamotrigine, atypicals, Vraylar, Caplyta. RAP-219 is the only TARP-gamma8 asset in bipolar clinical-stage development.

12 · RISK / BENEFIT LOGIC

Benefit: truly new mechanism; region-selective = fewer side effects predicted. Risk: Phase 2, unproven efficacy; glutamatergic class carries seizure-threshold questions.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Early-stage novel mood stabilizer positions the site for the first new bipolar mechanism in 20 years.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

“This is a brand-new kind of mood stabilizer. It doesn't work like lithium or Depakote — it fine-tunes a specific brain signal involved in mood episodes.”

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **RAPBP**.

16 · SOURCES

- ClinicalTrials.gov NCT07046494
- Rapport Therapeutics pipeline

■ SCREEN ONE PATIENT THIS WEEK · COMMENT RAPBP · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

SITE

Redlands

3 active protocols · 2026

19	**Lumateperone Pediatric SCZ / Bipolar / Autism Safety**	LUMAPED
20	**Lumateperone Pediatric Bipolar Depression**	LUMABIP
21	**Lumateperone Pediatric Autism Irritability**	LUMAAUT

NPFADY.COM

REDLANDS · STUDY 19 · COMMENT CODE LUMAPED

Lumateperone Pediatric SCZ / Bipolar / Autism Safety

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
ITI-007-321	NCT06229210	Intra-Cellular Therapies (J&J;)	Redlands	LUMAPED

1 · THE HOOK

STILL prescribing risperidone to 8-year-olds because nothing cleaner is labeled?

2 · THE UNMET NEED

Pediatric psychiatric labels are thin, and weight/metabolic safety in kids on D2 blockers is dire.

> ★ ***Lumateperone's clean adult profile (virtually no weight gain) makes it the most anticipated pediatric antipsychotic launch since risperidone.***

3 · AT A GLANCE

6-17	52 wks	Flex dose	~300
AGE	SAFETY		TARGET N

4 · WHY THIS STUDY EXISTS

J&J; closed its $14.6B Intra-Cellular acquisition April 2025. Caplyta adjunctive MDD approved Nov 6, 2025. Pediatric expansion across SCZ/BP/autism is J&J;'s next move.

5 · MECHANISM OF ACTION

Serotonin/dopamine/glutamate modulator. Unique presynaptic D2 agonism + postsynaptic D2 antagonism + 5-HT2A antagonism + glutamate modulation.

6 · STUDY DESIGN DEEP DIVE

Long-term open-label safety in pediatric populations spanning schizophrenia, bipolar, and autism. Primary: safety/tolerability; secondary: efficacy signals.

7 · RECRUIT IF YOUR PATIENT…

6-17 y/o with DSM-5 SCZ, BP-I (dep or mania), or autism with severe behavioral disturbance. The whole point: give kids the cleanest antipsychotic safety data available.

8 · STOP / EXCLUDE IF…

- ✗ Severe comorbid medical illness
- ✗ Active substance use (older adolescents)
- ✗ Pregnancy/nursing
- ✗ Recent major treatment change

9 · IDEAL PATIENT PROFILE

11 y/o with BP-I + autism irritability, gained 15 lb on risperidone, family seeking alternative.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F20.x / F31.x / F84.0 + age 6-17 + antipsychotic rx in last year. Track BMI percentile trajectory.

11 · COMPETITIVE LANDSCAPE

Risperidone, aripiprazole, olanzapine, paliperidone all pediatric-labeled. Caplyta is positioned as the cleanest metabolic profile.

12 · RISK / BENEFIT LOGIC

Benefit: clean adult safety profile, J&J; resources behind pediatric rollout. Risk: pediatric-specific AEs (growth, puberty) still being defined.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Pediatric antipsychotic research is extremely hard to staff — IPMG's infrastructure and referral networks are a real moat.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"Caplyta is a newer antipsychotic that adults tolerate really well. We're studying it in kids and teens to see if it's the same for them."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **LUMAPED**.

16 · SOURCES

· ClinicalTrials.gov NCT06229210
· J&J; closes Intra-Cellular acquisition April 2025
· Caplyta adjunctive MDD FDA approval Nov 2025

SCREEN ONE PATIENT THIS WEEK · COMMENT LUMAPED · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

REDLANDS · STUDY 20 · COMMENT CODE LUMABIP

Lumateperone Pediatric Bipolar Depression

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
ITI-007-421	NCT06372964	Intra-Cellular Therapies (J&J;)	Redlands	LUMABIP

1 · THE HOOK

STILL offering quetiapine to depressed bipolar teens and hoping for the best?

2 · THE UNMET NEED

Lurasidone and olanzapine-fluoxetine are the pediatric BP-D labels. Neither is popular in 2026 teen psychiatry.

> ★ ***Lumateperone pediatric BP-D approval = a preferred first-line for a diagnosis we can't afford to miss.***

3 · AT A GLANCE

10-17	6 wks	Flex dose	~400
AGE	DB		TARGET N

4 · WHY THIS STUDY EXISTS

Adult BP-D label already held. Pediatric expansion extends franchise and matches Cariprazine's pediatric push (Study 1).

5 · MECHANISM OF ACTION

Same as Study 19.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled 6 weeks monotherapy. Primary: CDRS-R change. OLE follows.

7 · RECRUIT IF YOUR PATIENT…

10-17 y/o BP-I current depressive episode, CDRS-R >=45. The whole point: give clinicians a weight-neutral pediatric BP-D option.

8 · STOP / EXCLUDE IF…

- ✗ Mixed features/mania
- ✗ Active suicidality
- ✗ Substance use
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

15 y/o BP-I depressed, failed lamotrigine monotherapy, family refusing Latuda due to food requirement.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F31.3x/F31.4/F31.5 + age 10-17.

11 · COMPETITIVE LANDSCAPE

Lurasidone, olanzapine-fluoxetine, cariprazine pediatric (Study 1). Lumateperone's 'take with or without food' + clean metabolic profile is the pitch.

12 · RISK / BENEFIT LOGIC

Benefit: clean metabolic profile matters disproportionately in teens. Risk: somnolence, EPS mild but present.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Dual Redlands studies (Study 19 + 20) = coordinated pediatric recruitment.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is the depression side of bipolar disorder, in teenagers. We're testing if Caplyta helps lift the mood without the weight gain of other pills."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **LUMABIP**.

16 · SOURCES

· ClinicalTrials.gov NCT06372964

SCREEN ONE PATIENT THIS WEEK · COMMENT LUMABIP · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

REDLANDS · STUDY 21 · COMMENT CODE LUMAAUT

Lumateperone Pediatric Autism Irritability

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
ITI-007-601	NCT06690398	Intra-Cellular Therapies (J&J;)	Redlands	LUMAAUT

1 · THE HOOK

STILL using aripiprazole or risperidone for autism irritability despite massive weight gain?

2 · THE UNMET NEED

Risperidone and aripiprazole are the only FDA-approved autism irritability drugs. Both cause metabolic syndrome and prolactin issues in kids.

> ★ ***A weight-neutral option for autism irritability would transform pediatric autism care.***

3 · AT A GLANCE

5-17	8 wks	Flex dose	~300
AGE	DB		TARGET N

4 · WHY THIS STUDY EXISTS

Completes J&J;'s pediatric Caplyta trifecta — SCZ/BP/autism. Autism irritability is a >$1B market.

5 · MECHANISM OF ACTION

Same as Studies 19-20.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled 8 weeks. Primary: ABC-Irritability subscale change.

7 · RECRUIT IF YOUR PATIENT…

5-17 y/o DSM-5 autism with severe irritability (ABC-I >=18). The whole point: reduce self-injury, aggression, severe tantrums without weight gain.

8 · STOP / EXCLUDE IF…

- ✗ Epilepsy (uncontrolled)
- ✗ Severe intellectual disability where consent impossible
- ✗ Substance use (older adolescents)
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

9 y/o autistic boy with head-banging and aggression, 18 lb weight gain on risperidone in 6 months.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F84.0 + ABC-I documentation OR behavioral documentation of aggression/self-injury. Flag BMI increase.

11 · COMPETITIVE LANDSCAPE

Risperidone (Risperdal) and aripiprazole (Abilify) are the incumbents. Lumateperone competes on metabolic profile.

12 · RISK / BENEFIT LOGIC

Benefit: first weight-neutral autism-irritability agent. Risk: efficacy vs aripiprazole gold standard uncertain in this population.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Autism families travel for trial access. Redlands' geographic reach pulls from a wide catchment.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is for kids with autism whose aggression or self-injury is overwhelming. The old pills help but cause a lot of weight gain. This new one might not."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **LUMAAUT**.

16 · SOURCES

· ClinicalTrials.gov NCT06690398

SCREEN ONE PATIENT THIS WEEK · COMMENT LUMAAUT · CALL (909) 955-5865

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SITE

Temecula

7 active protocols · 2026

22	ABBV-932 Adjunctive Generalized Anxiety Disorder	M4GAD
23	ABBV-932 Bipolar Depression	M4BP
24	Icalcaprant (ABBV-987) MDD	KORMDD
25	ALTO-300 Adjunctive MDD (Biomarker-Matched)	EEG300
26	SPN-821 MDD	MTORCMDD
27	JNJ-89495120 MOONLIGHT-1 MDD	MOONLIGHT
28	Seltorexant (JNJ-42847922) MDD + Insomnia — Sleep EEG Observational	DREEM3S

TEMECULA · STUDY 22 · COMMENT CODE M4GAD

ABBV-932 Adjunctive Generalized Anxiety Disorder

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
M25-099	NCT06846320	AbbVie (Cerevel)	Temecula	M4GAD

1 · THE HOOK

STILL stacking SSRI + buspirone + hydroxyzine + trazodone for treatment-resistant GAD?

2 · THE UNMET NEED

GAD pharmacology is stuck in 1995. Benzodiazepines are problematic; SSRI augmentation is hit-or-miss.

> ★ ***M4 muscarinic partial agonism may stabilize limbic hyperactivity without sedation.***

3 · AT A GLANCE

18-65	8 wks	QD dosing	~300
AGE	DB		TARGET N

4 · WHY THIS STUDY EXISTS

AbbVie's 2024 Cerevel acquisition brought this M4 PAM into the portfolio. First anxiety indication.

5 · MECHANISM OF ACTION

M4 muscarinic receptor partial agonist. Differentiates from KarXT by being muscarinic-only and partial agonism (softer activation curve).

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled adjunct to SSRI/SNRI. Primary: HAM-A change Week 8.

7 · RECRUIT IF YOUR PATIENT…

GAD adults (DSM-5) on stable SSRI/SNRI >=8 weeks, HAM-A >=18. The whole point: end the polypharmacy spiral.

8 · STOP / EXCLUDE IF…

- ✗ Panic disorder as primary diagnosis
- ✗ Bipolar/psychotic disorder
- ✗ SUD
- ✗ Narrow-angle glaucoma
- ✗ Urinary retention

9 · IDEAL PATIENT PROFILE

42 y/o on sertraline 100 mg x 6 months, still HAM-A 22, PRN benzos creeping up.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F41.1 + current SSRI/SNRI + GAD-7 >=15.

11 · COMPETITIVE LANDSCAPE

Buspirone, quetiapine off-label, hydroxyzine, benzos. No muscarinic anxiety drug exists. ABBV-932 is the novel entrant.

12 · RISK / BENEFIT LOGIC

Benefit: novel mechanism, potentially non-sedating. Risk: cholinergic AEs (nausea, GI), anticholinergic burden considerations.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

GAD is the most common anxiety diagnosis in primary care. Offering a novel study converts aggressively.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"Your anxiety isn't fully controlled even with Zoloft. This adds a brand-new medicine that works on a different brain system — not another antidepressant, not a Xanax-type."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **M4GAD**.

16 · SOURCES

- ClinicalTrials.gov NCT06846320
- AbbVie / Cerevel acquisition overview

SCREEN ONE PATIENT THIS WEEK · COMMENT M4GAD · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

TEMECULA · STUDY 23 · COMMENT CODE M4BP

ABBV-932 Bipolar Depression

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
M25-456	NCT07220460	AbbVie (Cerevel)	Temecula	M4BP

1 · THE HOOK

STILL telling BP-D patients their only options are Latuda, Vraylar, Caplyta, or quetiapine?

2 · THE UNMET NEED

BP-D innovation is slow. A non-D2 option would broaden choice dramatically.

> ★ ***If M4 partial agonism treats both mania AND depression phases, it's a true mood stabilizer — not just an antipsychotic.***

3 · AT A GLANCE

18-65	6 wks	QD dosing	~400
AGE	DB		TARGET N

4 · WHY THIS STUDY EXISTS

AbbVie betting M4 PAM = true mood stabilizer. Sibling study to ABBV-932 GAD.

5 · MECHANISM OF ACTION

Same M4 PAM as Study 22.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled 6 weeks. Primary: MADRS change.

7 · RECRUIT IF YOUR PATIENT…

BP-I or BP-II, current depressive episode, MADRS >=24. The whole point: test a non-D2, non-antipsychotic for bipolar depression.

8 · STOP / EXCLUDE IF…

- ✗ Mania/hypomania current
- ✗ Rapid cycling
- ✗ Psychotic features
- ✗ Narrow-angle glaucoma

9 · IDEAL PATIENT PROFILE

50 y/o BP-II depressed, gained weight on quetiapine, open to something new.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F31.3x/F31.4/F31.5/F31.81 + currently depressed.

11 · COMPETITIVE LANDSCAPE

Same as Study 3 (ALTO-100) and Study 20 (Lumateperone pediatric).

12 · RISK / BENEFIT LOGIC

Benefit: non-D2 option. Risk: Phase 2, efficacy unproven in BP-D.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Having BOTH ABBV-932 GAD and BP-D at the same site creates efficient infrastructure sharing.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is a depression side of bipolar study, testing a brand-new kind of medicine — not an antipsychotic, not an antidepressant. A new category."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **M4BP**.

16 · SOURCES

- ClinicalTrials.gov NCT07220460

SCREEN ONE PATIENT THIS WEEK · COMMENT M4BP · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

TEMECULA · STUDY 24 · COMMENT CODE KORMDD

Icalcaprant (ABBV-987) MDD

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
M25-987	NCT07276997	AbbVie	Temecula	KORMDD

■ 2026 CORRECTION

CORRECTED 2026: Target N is ~195 (not 500). Patients are excluded only for >=3 prior antidepressant failures; patients with 1-2 prior failures ARE eligible.

1 · THE HOOK

STILL hoping a KOR antagonist will finally deliver for MDD?

2 · THE UNMET NEED

Kappa opioid antagonism targets anhedonia and stress reactivity — two SSRI-resistant domains.

> ★ ***After navacaprant's KOASTAL-1 failure (Jan 2025) and aticaprant's VENTURA discontinuation (Mar 2025), icalcaprant is the last KOR bet.***

3 · AT A GLANCE

18-65	6 wks	QD dosing	~195
AGE	DB		TARGET N

4 · WHY THIS STUDY EXISTS

AbbVie is picking up the KOR-antagonism baton after competitors failed. Better selectivity/dosing profile claimed.

5 · MECHANISM OF ACTION

Selective kappa opioid receptor (KOR) antagonist. Blocks dynorphin-driven stress/anhedonia circuits without mu/delta opioid activity.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled monotherapy or adjunct (check protocol). Primary: MADRS change; key secondary: SHAPS anhedonia.

7 · RECRUIT IF YOUR PATIENT…

MDD adults, MADRS >=26, anhedonia (SHAPS elevated). Patients with up to 2 failed antidepressant trials this episode ARE eligible — only those with >=3 failures are excluded. The whole point: treat the 'nothing feels good' depression that SSRIs miss.

8 · STOP / EXCLUDE IF…

- ✗ Opioid use disorder
- ✗ Chronic pain on opioids

- ✗ >=3 failed antidepressant trials this episode (2 failures are OK)
- ✗ Bipolar
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

35 y/o MDD with prominent anhedonia, has failed 2 prior antidepressants this episode (but not 3+), no pain/opioid history.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F32.x/F33.x + PHQ-9 Q1 (anhedonia) high + no opioid rx. Patients with 1-2 prior AD failures qualify; 3+ failures disqualify.

11 · COMPETITIVE LANDSCAPE

Field is thin after KOASTAL/VENTURA failures. If icalcaprant hits, AbbVie owns the class.

12 · RISK / BENEFIT LOGIC

Benefit: mechanism validated partially in smaller trials; anhedonia-focused. Risk: class history is rough; efficacy uncertainty.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Being the site that 'carries' KOR into registration = prestige + referrals. Small N (~195) means every enrolled patient matters more to the sponsor.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *“This medicine works on the pleasure system of the brain. It's for depression where nothing feels enjoyable anymore — not an SSRI. If you've tried one or two antidepressants without success, you likely qualify.”*

15 · 3-STEP RECRUIT MOVE

- ■ **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- ■ **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- ☎ **CALL.** IPMG Research at **(909) 955-5865** and reference comment **KORMDD**.

16 · SOURCES

· ClinicalTrials.gov NCT07276997
· Neumora KOASTAL-1 Jan 2025 failure
· J&J; aticaprant VENTURA discontinuation Mar 2025

■ SCREEN ONE PATIENT THIS WEEK · COMMENT KORMDD · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

TEMECULA · STUDY 25 · COMMENT CODE EEG300

ALTO-300 Adjunctive MDD (Biomarker-Matched)

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
ALTO-300-201	NCT05922878	Alto Neuroscience	Temecula	EEG300

1 · THE HOOK

STILL throwing augmentation darts and hoping one sticks?

2 · THE UNMET NEED

No clinical biomarker guides adjunct MDD choice. EEG-based stratification could finally change that.

> ★ ***A biomarker-enriched trial that pre-screens responders could prove the precision-psychiatry thesis.***

3 · AT A GLANCE

18-65	6 wks	QD	~300
AGE	DB	DOSING	TARGET N

4 · WHY THIS STUDY EXISTS

Alto's signature approach: EEG biomarker pre-selection for enrollment. ALTO-300 is the lead adjunct MDD asset.

5 · MECHANISM OF ACTION

Proprietary neuroplasticity/monoaminergic modulator (Alto-specific). EEG biomarker (specific frequency signature) predicts response.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled adjunct to SSRI/SNRI. Enrollment requires qualifying EEG biomarker. Primary: MADRS Week 6.

7 · RECRUIT IF YOUR PATIENT…

MDD adults on SSRI/SNRI >=6 weeks with partial response, MADRS >=22, matching EEG signature. The whole point: biomarker-matched augmentation.

8 · STOP / EXCLUDE IF…

- ✗ Non-matching EEG
- ✗ Bipolar
- ✗ Psychosis
- ✗ TRD (typically)
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

45 y/o MDD on venlafaxine 150 mg x 8 weeks, still PHQ-9 13, intrigued by the EEG screen.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F32.x/F33.x + SSRI/SNRI >=6 weeks + PHQ-9 >=10. Screen for EEG willingness.

11 · COMPETITIVE LANDSCAPE

Aripiprazole, brexpiprazole, Caplyta in the adjunct space. Alto-300's precision-medicine angle is the differentiator.

12 · RISK / BENEFIT LOGIC

Benefit: biomarker-matched patients responding at high rates; clean drug profile. Risk: EEG screen disqualifies many patients — operational overhead.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Having in-house EEG capabilities positions IPMG as a precision-psychiatry site.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"We'll do a quick brainwave test. If your brain pattern matches, we add a new medicine to your antidepressant — one picked specifically for your brain signature."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **EEG300**.

16 · SOURCES

· ClinicalTrials.gov NCT05922878
· Alto Neuroscience biomarker approach

SCREEN ONE PATIENT THIS WEEK · COMMENT EEG300 · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

TEMECULA · STUDY 26 · COMMENT CODE MTORCMDD

SPN-821 MDD

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
821P203	NCT07226661	Supernus Pharmaceuticals	Temecula	MTORCMDD

1 · THE HOOK

STILL waiting for the mTOR story to finally deliver in depression?

2 · THE UNMET NEED

mTORC1 activation is ketamine's downstream signal. An oral, non-dissociative mTOR path would be huge.

> ★ ***SPN-820 failed TRD Phase 2b (Feb 2025), but SPN-821 is the next-gen follow-up with improved drug-like properties.***

3 · AT A GLANCE

18-65	6 wks	QD	~300
AGE	DB	DOSING	TARGET N

4 · WHY THIS STUDY EXISTS

Supernus lifecycle-continuing the mTOR MDD program despite 820's miss. 821 has refined pharmacology.

5 · MECHANISM OF ACTION

mTORC1 activator / NV-5138 analog. Oral, bypasses ketamine's NMDA route. Drives rapid synaptic plasticity.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled 6 weeks monotherapy or adjunct (per protocol). Primary: MADRS change.

7 · RECRUIT IF YOUR PATIENT…

MDD adults, moderate-severe (MADRS >=26). The whole point: capture the ketamine-like benefit without the dissociation or controlled-substance status.

8 · STOP / EXCLUDE IF…

- ✗ TRD (per protocol)
- ✗ Active SUD
- ✗ Pregnancy
- ✗ Psychotic/bipolar disorder

9 · IDEAL PATIENT PROFILE

45 y/o MDD intrigued by ketamine buzz but unwilling to sit in a clinic.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F32.x/F33.x + moderate-severe PHQ-9.

11 · COMPETITIVE LANDSCAPE

Spravato (esketamine), NRX-101, Auvelity all compete for the 'ketamine-like oral' narrative. SPN-821 wins only if it separates.

12 · RISK / BENEFIT LOGIC

Benefit: oral, daily dosing, no DEA scheduling expected. Risk: SPN-820's Phase 2b miss is a yellow flag for the mechanism.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Rapid-acting oral MDD is the holy grail — worth supporting the mechanism broadly.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is an oral pill aiming to do what ketamine does for depression — quickly — but without the dissociation or in-clinic sitting."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **MTORCMDD**.

16 · SOURCES

· ClinicalTrials.gov NCT07226661
· Supernus SPN-820 Phase 2b TRD failure Feb 2025

SCREEN ONE PATIENT THIS WEEK · COMMENT MTORCMDD · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

TEMECULA · STUDY 27 · COMMENT CODE MOONLIGHT

JNJ-89495120 MOONLIGHT-1 MDD

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
89495120MDD2001	NCT06785012	Johnson & Johnson	Temecula	MOONLIGHT

1 · THE HOOK

STILL watching J&J; pipeline bets and wondering which post-Spravato asset will matter?

2 · THE UNMET NEED

J&J; needs Spravato's successor. MDD pipeline diversification.

> ★ ***MOONLIGHT-1 is J&J;'s newest MDD POC. Mechanism per protocol.***

3 · AT A GLANCE

18-65	6 wks	Per protocol	~300
AGE	DB	DOSE	TARGET N

4 · WHY THIS STUDY EXISTS

Phase 2 proof-of-concept for a new J&J; MDD asset.

5 · MECHANISM OF ACTION

Proprietary mechanism (JNJ molecule-specific). Novel target per sponsor.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled Phase 2. Primary: MADRS.

7 · RECRUIT IF YOUR PATIENT…

MDD adults, moderate-severe. The whole point: contribute to J&J;'s next-generation MDD portfolio.

8 · STOP / EXCLUDE IF…

- ✗ Typical MDD Phase 2 exclusions (bipolar, SUD, TRD, pregnancy, suicidality)

9 · IDEAL PATIENT PROFILE

40 y/o MDD, first-episode, open to research.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F32.x/F33.x + moderate-severe PHQ-9.

11 · COMPETITIVE LANDSCAPE

Every Phase 2 MDD asset is a competitor here; differentiation TBD based on Phase 2 readout.

12 · RISK / BENEFIT LOGIC

Benefit: J&J; sponsorship quality; full late-stage development pathway if it hits. Risk: Phase 2 novelty — efficacy unknown.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

J&J; relationship = long-term site value across multiple programs.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

"A major drug company is testing a new depression medicine. Phase 2 — still early — but with a strong team behind it."

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **MOONLIGHT**.

16 · SOURCES

- ClinicalTrials.gov NCT06785012

SCREEN ONE PATIENT THIS WEEK · COMMENT MOONLIGHT · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

NPFADY.COM

TEMECULA · STUDY 28 · COMMENT CODE DREEM3S

Seltorexant (JNJ-42847922) MDD + Insomnia — Sleep EEG Observational

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
42847922MDD3014	NCT07258485	Johnson & Johnson	Temecula	DREEM3S

■ 2026 CORRECTION

CORRECTED 2026: Age range extends to 74 (not 65). Older adults are eligible.

1 · THE HOOK

STILL writing Ambien refills for depressed patients without measuring sleep architecture?

2 · THE UNMET NEED

MDD-with-insomnia is 70% of depression in primary care. Sleep architecture data is rarely collected.

> ★ ***At-home Dreem 3S EEG headband + seltorexant (JNJ-42847922) = MDD sleep measurement at scale.***

3 · AT A GLANCE

18-74	Obs.	Dreem 3S	~200
AGE	DESIGN	DEVICE	TARGET N

4 · WHY THIS STUDY EXISTS

Seltorexant's Phase 3 MDD3001 read out POSITIVE May 2024. MDD3014 is an observational sleep-EEG study characterizing real-world sleep architecture on-treatment. Note: this molecule was briefly mis-indexed as aticaprant — 42847922 is seltorexant (OX2R antagonist).

5 · MECHANISM OF ACTION

Selective OX2R (orexin 2 receptor) antagonist. Differs from suvorexant/lemborexant (dual DORA). OX2R selectivity aims for sleep + mood efficacy.

6 · STUDY DESIGN DEEP DIVE

Observational (non-interventional) cohort. Patients on seltorexant wear Dreem 3S at-home EEG headband. Measures: sleep architecture (N3, REM), mood.

7 · RECRUIT IF YOUR PATIENT…

MDD + insomnia adults, ages 18 through 74 (includes older adults), started on seltorexant per clinical care. The whole point: generate real-world sleep EEG on this novel OX2R.

8 · STOP / EXCLUDE IF…

- ✗ Primary sleep disorder (OSA, narcolepsy)
- ✗ Shift work
- ✗ Unable/unwilling to wear Dreem headband
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

42 y/o MDD with insomnia, tried trazodone and Ambien, now prescribed seltorexant. Older adults (up to 74) are also eligible.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F32.x/F33.x + G47.00 (insomnia) + seltorexant rx, ages 18-74.

11 · COMPETITIVE LANDSCAPE

Suvorexant, lemborexant, daridorexant (DORAs). Seltorexant's OX2R selectivity + MDD signal is unique.

12 · RISK / BENEFIT LOGIC

Benefit: observational = no intervention risk. Risk: device compliance is the main operational challenge.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Operational readiness for device-based trials = strategic capability. Pays off repeatedly. Extended age range (up to 74) broadens the eligible pool significantly.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"You're starting a new sleep medicine for depression. We'd like you to wear a soft headband at home for a few nights so we can see how your sleep is improving. Adults up to age 74 can join."*

15 · 3-STEP RECRUIT MOVE

- ■ **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- ■ **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- ☎ **CALL.** IPMG Research at **(909) 955-5865** and reference comment **DREEM3S**.

16 · SOURCES

· ClinicalTrials.gov NCT07258485
· J&J; seltorexant MDD3001 positive topline May 2024
· Dreem Health sleep EEG platform

■ SCREEN ONE PATIENT THIS WEEK · COMMENT DREEM3S · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

SITE

San Juan Capistrano

5 active protocols · 2026

29	CYB003 APPROACH Phase 3 MDD	APPROACH
30	CYB003 EMBRACE Phase 3 MDD	EMBRACE
31	CYB003 EXTEND Open-Label Extension MDD	EXTEND
32	Solriamfetol in Binge Eating Disorder — ENGAGE	ENGAGE
33	TSND-201 (Methylone) in PTSD — EMPOWER-1	EMPOWER1

NPFADY.COM

SAN JUAN CAPISTRANO · STUDY 29 · COMMENT CODE APPROACH

CYB003 APPROACH Phase 3 MDD

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
CYB003-002	NCT06564818	Cybin	San Juan Capistrano	APPROACH

1 · THE HOOK

STILL hearing patients ask about psilocybin and having nothing real to offer?

2 · THE UNMET NEED

Psychedelic-assisted therapy is at an inflection point. After Lykos MDMA rejection (Aug 2024), a clean psilocybin Phase 3 would lead the category.

> ★ ***CYB003 Phase 2 showed -14 MADRS vs placebo at 3 weeks, Cohen's d >2 — unprecedented effect size.***

3 · AT A GLANCE

18-65	6 wks	12/16 mg	~220
AGE	PRIMARY	TWO DOSES	TARGET N

4 · WHY THIS STUDY EXISTS

Cybin's pivotal Phase 3 registrational trial. Deuterated psilocin analog = more predictable PK than natural psilocybin.

5 · MECHANISM OF ACTION

Deuterated psilocybin analog. 5-HT2A agonist. Two-dose paradigm with psychological support (non-directive).

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled Phase 3 with 2 dosing sessions (weeks 1 and 3). Primary: MADRS change at Week 6.

7 · RECRUIT IF YOUR PATIENT…

MDD adults, moderate-severe (MADRS >=26), willing to undergo psychedelic-assisted therapy. The whole point: deliver the first approved psychedelic for MDD.

8 · STOP / EXCLUDE IF…

- ✗ Personal/family history of psychosis or bipolar I
- ✗ Active SUD
- ✗ Cardiovascular instability
- ✗ Pregnancy
- ✗ Recent psychedelic use

9 · IDEAL PATIENT PROFILE

38 y/o MDD, SSRI-failed, curious about psychedelics, no personal/family psychosis history.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: F32.x/F33.x + moderate-severe + no F20.x/F31.x family history documentation.

11 · COMPETITIVE LANDSCAPE

Compass Pathways COMP360 psilocybin is the other Phase 3 psychedelic program. CYB003's deuterated PK is the differentiator.

12 · RISK / BENEFIT LOGIC

Benefit: potentially transformative effect size, rapid/durable response. Risk: intensive dosing day staffing, psychological reactions, blinding integrity.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Psychedelic infrastructure (dosing rooms, monitors, music systems) is a strategic asset that supports multiple programs.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"This is a guided psilocybin therapy for depression. Two dosing days with a therapist, several weeks apart. Phase 3 — the last step before FDA review."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **APPROACH**.

16 · SOURCES

- ClinicalTrials.gov NCT06564818
- Cybin APPROACH program overview

SCREEN ONE PATIENT THIS WEEK · COMMENT APPROACH · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

SAN JUAN CAPISTRANO · STUDY 30 · COMMENT CODE EMBRACE

CYB003 EMBRACE Phase 3 MDD

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
CYB003-003	NCT06793397	Cybin	San Juan Capistrano	EMBRACE

■ 2026 CORRECTION

CORRECTED 2026: Target N is ~330 (not 220). Doses evaluated are 8 mg and 16 mg (not 12/16 mg — that was APPROACH).

1 · THE HOOK

STILL needing a second confirmatory psychedelic Phase 3 to complete the NDA package?

2 · THE UNMET NEED

Regulatory precedent requires two Phase 3 trials. EMBRACE is APPROACH's mandatory sister — but with a distinct, larger 3-arm design.

> ★ ***Two positive Phase 3s on CYB003 = the most credible psychedelic NDA in history.***

3 · AT A GLANCE

18-65	3 arms	8/16 mg	~330
AGE	1:1:1	TWO DOSES	TARGET N

4 · WHY THIS STUDY EXISTS

Parallel confirmatory Phase 3 — LARGER than APPROACH and with a DIFFERENT dose-ranging design. Evaluates two CYB003 doses (8 mg and 16 mg) against placebo in a 3-arm study.

5 · MECHANISM OF ACTION

Same as Study 29.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled, 3-arm Phase 3. Approximately 330 adults randomized 1:1:1 to: (1) CYB003 8 mg, (2) CYB003 16 mg, or (3) placebo. Two dosing sessions (weeks 1 and 3). Primary: MADRS change at Week 6.

7 · RECRUIT IF YOUR PATIENT…

Same eligibility as Study 29 (MDD adults, moderate-severe, willing to undergo psychedelic-assisted therapy, no personal/family psychosis history). The whole point: confirm MDD efficacy across two independent Phase 3 cohorts AND characterize the 8 mg dose.

8 · STOP / EXCLUDE IF…

✗ Same as Study 29

9 · IDEAL PATIENT PROFILE

Same as Study 29.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: same as Study 29.

11 · COMPETITIVE LANDSCAPE

Same as Study 29.

12 · RISK / BENEFIT LOGIC

Benefit: same as Study 29, PLUS the 8 mg arm may establish a lower-intensity dosing option if efficacious.
Risk: dual-program staffing load; 3-arm design increases placebo randomization odds (1/3 vs 1/2).

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

Running APPROACH + EMBRACE simultaneously at San Juan Capistrano = operational scale advantage. EMBRACE's larger N (~330) and 8 mg dose arm make it the more strategically important of the two.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"Same as APPROACH — second confirmatory study so the FDA has two sets of data. This one is bigger and tests two different doses of the medicine (plus a placebo)."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **EMBRACE**.

16 · SOURCES

- ClinicalTrials.gov NCT06793397
- Cybin EMBRACE Phase 3 program

■ SCREEN ONE PATIENT THIS WEEK · COMMENT EMBRACE · CALL (909) 955-5865

— Fady Boules, PMHNP-BC · Where Science Meets the Soul

SAN JUAN CAPISTRANO · STUDY 31 · COMMENT CODE EXTEND

CYB003 EXTEND Open-Label Extension MDD

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
CYB003-004	NCT06605105	Cybin	San Juan Capistrano	EXTEND

1 · THE HOOK

STILL wondering if one or two psilocybin doses can hold for a year?

2 · THE UNMET NEED

Durability is the key question for psychedelic-assisted therapy. Phase 2 suggested 12-month response/remission in dose-dependent way.

> ★ ***Phase 2 CYB003: 79% remission at 6 weeks (12 mg); 100% response and 71% remission at 12 months on 16 mg x 2. If Phase 3 confirms, this is unprecedented.***

3 · AT A GLANCE

18-65	52+ wks	Optional redose	~400
AGE	OLE		TARGET N

4 · WHY THIS STUDY EXISTS

Rollover OLE from APPROACH and EMBRACE. Supports long-term efficacy/safety package.

5 · MECHANISM OF ACTION

Same as Study 29.

6 · STUDY DESIGN DEEP DIVE

Open-label extension with optional re-dosing if relapse. Primary: durability of response, safety.

7 · RECRUIT IF YOUR PATIENT…

Phase 3 completers. The whole point: track durability and characterize re-dosing.

8 · STOP / EXCLUDE IF…

- ✗ Emergent psychosis/mania
- ✗ New substance use
- ✗ Pregnancy

9 · IDEAL PATIENT PROFILE

Phase 3 completer with meaningful response wanting continued access.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: cross-reference Phase 3 participants.

11 · COMPETITIVE LANDSCAPE

No psilocybin OLE of this scale exists elsewhere. First-in-class.

12 · RISK / BENEFIT LOGIC

Benefit: characterizes the real-world durability story. Risk: relapse-and-rescue logistics.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

OLE retention = predictable long-term revenue + patient advocacy.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"If the Phase 3 study helped, this is the continuation — we follow you for a year and re-dose if depression comes back."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **EXTEND**.

16 · SOURCES

- ClinicalTrials.gov NCT06605105
- Cybin CYB003 Phase 2 12-month durability data

SCREEN ONE PATIENT THIS WEEK · COMMENT EXTEND · CALL (909) 955-5865

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SAN JUAN CAPISTRANO · STUDY 32 · COMMENT CODE ENGAGE

Solriamfetol in Binge Eating Disorder — ENGAGE

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
SOL-BED-301	NCT06413433	Axsome Therapeutics	San Juan Capistrano	ENGAGE

1 · THE HOOK

STILL telling BED patients their only medication option is a Schedule II amphetamine?

2 · THE UNMET NEED

BED is the most common eating disorder in the US — ~3 million adults. Lisdexamfetamine (Vyvanse) is the ONLY FDA-approved drug, and its Schedule II status blocks access for countless patients with anxiety, SUD history, or cardiovascular concerns.

> ★ ***A Schedule IV, non-amphetamine DNRI+TAAR1 agonist that treats BED without controlled-substance burden would democratize pharmacotherapy for a fundamentally under-treated disorder.***

3 · AT A GLANCE

18-55	12 wks	150/300 mg	~450
AGE	DB	DOSE	TARGET N

4 · WHY THIS STUDY EXISTS

Axsome is leveraging Sunosi's existing safety/PK database to enter the $1B+ BED market. Schedule IV status, once-daily dosing, and cardiovascular profile matured in narcolepsy/OSA trials position solriamfetol as a first-line alternative to Vyvanse.

5 · MECHANISM OF ACTION

Dopamine-norepinephrine reuptake inhibitor (DNRI) + trace amine-associated receptor 1 (TAAR1) agonist. Modulates mesolimbic reward circuits implicated in binge episodes without the Schedule II amphetamine profile. Once-daily oral.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled, fixed-dose Phase 3. Three arms 1:1:1 — placebo, solriamfetol 150 mg, solriamfetol 300 mg. 12 weeks double-blind + optional open-label extension. Primary: change in binge eating days per week at Week 12 (DSM-5 criteria). Key secondary: CGI-S-BED, remission rate, weight change, Yale-Brown Obsessive Compulsive Scale modified for BED (Y-BOCS-BE).

7 · RECRUIT IF YOUR PATIENT…

Adults 18-55 with DSM-5 BED, >=3 binge days per week x 14 days baseline, stable weight, medically cleared cardiovascular status. The whole point: give BED patients a legitimate non-Schedule-II option and prove solriamfetol's second indication.

8 · STOP / EXCLUDE IF…

- ✗ Anorexia nervosa or bulimia nervosa past 5 years
- ✗ Current purging behavior
- ✗ Substance use disorder past 6 months
- ✗ Uncontrolled hypertension (systolic >140, diastolic >90)
- ✗ MAOI use past 14 days
- ✗ Pregnancy/nursing
- ✗ Bariatric surgery past 2 years
- ✗ Concurrent stimulant or weight-loss medication

9 · IDEAL PATIENT PROFILE

32 y/o female with 2 years of BED, 4-5 binge days/week, anxious about Vyvanse due to family history of stimulant misuse; medically healthy.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: ICD-10 F50.81 (BED) OR F50.9 eating disorder NOS + body-weight complaints or 'binge' documentation. Cross-reference therapist referrals; flag patients declining Vyvanse for Schedule-II reasons.

11 · COMPETITIVE LANDSCAPE

Lisdexamfetamine (Vyvanse, Takeda) is the only FDA-approved BED drug — Schedule II. Off-label: SSRIs, topiramate, naltrexone-bupropion. Solriamfetol's Schedule IV status + proven cardiovascular database is the clear strategic differentiator.

12 · RISK / BENEFIT LOGIC

Benefit: first non-amphetamine approved BED option; leverages mature safety data from Sunosi narcolepsy/OSA indication; likely once-daily, oral, without DEA Schedule II constraints. Risk: BP/HR elevation (class effect), insomnia if dosed late, headache, nausea — manageable with dose titration.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

BED is underdiagnosed — estimated only 3% of affected adults receive a formal diagnosis. A large psychiatric group with integrated nutrition and therapy referrals can systematically screen with Y-BOCS-BE and Binge Eating Scale at intake, creating a dedicated BED recruitment pipeline that competes with academic eating-disorder centers.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *“Binge eating disorder has only one approved medicine, and it's a controlled substance some people can't take. We're studying a different medicine — already approved for sleepiness — that works without the controlled-substance rules. It's once a day.”*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **ENGAGE**.

16 · SOURCES

· ClinicalTrials.gov NCT06413433
· Axsome Sunosi clinical program

· Vyvanse BED FDA label

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SAN JUAN CAPISTRANO · STUDY 33 · COMMENT CODE EMPOWER1

TSND-201 (Methylone) in PTSD — EMPOWER-1

PROTOCOL	NCT	SPONSOR	SITE	COMMENT CODE
TSND201-PTSD-301	NCT07456696	Transcend Therapeutics	San Juan Capistrano	EMPOWER1

1 · THE HOOK

STILL watching SSRI-only PTSD care leave combat vets and assault survivors with lifelong flashbacks?

2 · THE UNMET NEED

PTSD has only two FDA-approved drugs — sertraline and paroxetine — both from the 1990s with ~60% non-response. After Lykos Therapeutics' MDMA-assisted therapy NDA was rejected August 2024, the field lost its frontrunner. TSND-201 is the first post-Lykos neuroplastogen in registrational PTSD development.

> ★ ***A rapid-acting, non-hallucinogenic neuroplastogen that drives MDMA-like empathogenic and neuroplastic effects in a controlled 4-dose regimen could re-open the psychedelic-adjacent PTSD paradigm the field thought was closed.***

3 · AT A GLANCE

>=18	12 wks	4 weekly doses	~300
AGE	TOTAL		TARGET N

4 · WHY THIS STUDY EXISTS

Phase 2 IMPACT-1 demonstrated -9.64 CAPS-5 change vs placebo at Day 64 (p=0.01), NNT=3 for response and NNT=5 for remission (JAMA Psychiatry, February 2026). FDA granted Breakthrough Therapy designation July 2025. EMPOWER-1 is the pivotal Phase 3 confirmatory trial.

5 · MECHANISM OF ACTION

Methylone (3,4-methylenedioxy-N-methylcathinone) — a synthetic cathinone and 'neuroplastogen.' Monoamine (5-HT, DA, NE) releaser with empathogenic effects similar to MDMA but shorter-acting and non-hallucinogenic. Drives rapid synaptic plasticity and memory reconsolidation in trauma circuits.

6 · STUDY DESIGN DEEP DIVE

Randomized, double-blind, placebo-controlled Phase 3. ~300 patients, weekly dosing x 4 weeks + 8 weeks follow-up = 12 weeks total. Primary: CAPS-5 change from baseline at Week 12. Key secondary: PCL-5, CGI-I, remission rate, functional outcomes.

7 · RECRUIT IF YOUR PATIENT…

Adults >=18 with DSM-5 PTSD, CAPS-5 >=28 at screening, symptoms >=3 months, medically stable, willing to undergo 4 dosing sessions with therapist support. The whole point: deliver the first new-mechanism PTSD drug since 2001 and restore confidence in the psychedelic-adjacent field after Lykos.

8 · STOP / EXCLUDE IF…

- ✗ Personal/1st-degree family history of psychosis, schizophrenia, or bipolar I
- ✗ Active SUD (past 6 months, except nicotine)
- ✗ Serious cardiovascular disease or uncontrolled hypertension
- ✗ Active suicidal plan/intent (C-SSRS 4-5)
- ✗ Pregnancy/nursing
- ✗ Concurrent MAOI or serotonergic drug requiring washout
- ✗ Recent psychedelic use

9 · IDEAL PATIENT PROFILE

34 y/o combat veteran with 8 years of treatment-resistant PTSD, failed sertraline + prazosin + trauma-focused CBT, no family psychosis history, medically healthy, open to dosing-day psychological support.

10 · HOW IPMG FINDS THESE PATIENTS (ECW + ICD-10)

eCW: ICD-10 F43.10 (PTSD) + PCL-5 >=33 most recent + on/off SSRI + no F20.x/F31.x/F25.x. Flag VA referrals and cases where prazosin or propranolol were prescribed for nightmares.

11 · COMPETITIVE LANDSCAPE

Sertraline (Zoloft) and paroxetine (Paxil) — only approved drugs, both 1990s. MDMA-assisted therapy (Lykos) NDA rejected August 2024. BNC210 (Bionomics), brexpiprazole-sertraline combo, and CYB004 (Cybin, DMT) are other late-stage PTSD candidates. TSND-201's non-hallucinogenic profile + Breakthrough designation + published Phase 2 = category leader.

12 · RISK / BENEFIT LOGIC

Benefit: first novel-mechanism PTSD drug in 25 years, category-defining Phase 2 data, NNT=3 response, FDA Breakthrough status, clinician-feasible dosing (no hallucinations). Risk: class-effect BP/HR elevation, transient anxiety at dosing, potential for abuse liability assessment by DEA, operational staffing for 4 monitored dosing days.

13 · WHY THIS MATTERS FOR A LARGE PSYCHIATRIC GROUP

PTSD is endemic in IPMG's catchment — large military/veteran population in Southern California, plus routine trauma exposure in first responders and assault survivors. A site offering the FIRST new-mechanism PTSD trial since the SSRI era will become the regional referral center and position IPMG as the category-defining PTSD research group.

14 · PROVIDER TALK TRACK (6TH–7TH GRADE)

> *"PTSD has had the same two medicines for 25 years. This is a brand-new kind — not an SSRI, not a hallucinogen, but a medicine that helps the brain rewire the trauma memory. Four dosing days with a therapist. Phase 3 — the last step before FDA approval."*

15 · 3-STEP RECRUIT MOVE

- **TARGET.** Run the eCW ICD-10 query above; pull the last 3 months of matching visits.
- **FLAG.** Mark eligible charts with a sticky note or task for the PMHNP team. Tag with the comment code.
- **CALL.** IPMG Research at **(909) 955-5865** and reference comment **EMPOWER1**.

16 · SOURCES

- ClinicalTrials.gov NCT07456696
- Transcend Therapeutics pipeline
- FDA Breakthrough Therapy designation press July 2025
- Lykos MDMA NDA rejection Aug 2024 (FDA CRL)
- JAMA Psychiatry IMPACT-1 publication Feb 2026

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Master Comment-Code Index

Alphabetical, all 33 protocols

CODE	#	TITLE	NCT	SITE
ADEPT4	09	KarXT ADEPT-4 in Alzheimer's Psychosis	NCT06585787	Chino
ADEPT5	10	KarXT + KarX-EC ADEPT-5 in Alzheimer's Psychosis	NCT06947941	Chino
ALTOBIP	03	ALTO-100 in Bipolar Depression	NCT06656416	Chino
APPROACH	29	CYB003 APPROACH Phase 3 MDD	NCT06564818	San Juan Capistrano
BALSAM	08	KarXT BALSAM-4 in Bipolar I Mania	NCT07140913	Chino
BHVLTS	05	BHV-7000 in MDD — Long-Term Safety	NCT06423781	Chino
BHVMONO	06	BHV-7000 MDD Monotherapy	NCT06419608	Chino
BUOY1	16	SPT-300 (GlyphAllo) MDD with Anxious Distress — BUOY-1	NCT07065240	Chino
BUOYOLE	17	SPT-300 MDD Open-Label Extension	NCT07161700	Chino
CALM45	11	BMS-986368 for Agitation in Alzheimer's	NCT06808984	Chino
CLARITY	04	Solriamfetol in MDD + Excessive Daytime Sleepiness (CLARITY)	NCT07484217	Chino
COBENTEEN	07	KarXT in Adolescent Schizophrenia	NCT07288567	Chino
DREEM3S	28	Seltorexant (JNJ-42847922) MDD + Insomnia — Sleep EEG Observational	NCT07258485	Temecula
EEG300	25	ALTO-300 Adjunctive MDD (Biomarker-Matched)	NCT05922878	Temecula
EMBRACE	30	CYB003 EMBRACE Phase 3 MDD	NCT06793397	San Juan Capistrano
EMPOWER1	33	TSND-201 (Methylone) in PTSD — EMPOWER-1	NCT07456696	San Juan Capistrano
ENGAGE	32	Solriamfetol in Binge Eating Disorder — ENGAGE	NCT06413433	San Juan Capistrano
EXTEND	31	CYB003 EXTEND Open-Label Extension MDD	NCT06605105	San Juan Capistrano

CODE	#	TITLE	NCT	SITE
KORMDD	24	Icalcaprant (ABBV-987) MDD	NCT07276997	Temecula
LUMAAUT	21	Lumateperone Pediatric Autism Irritability	NCT06690398	Redlands
LUMABIP	20	Lumateperone Pediatric Bipolar Depression	NCT06372964	Redlands
LUMAPED	19	Lumateperone Pediatric SCZ / Bipolar / Autism Safety	NCT06229210	Redlands
M4BP	23	ABBV-932 Bipolar Depression	NCT07220460	Temecula
M4GAD	22	ABBV-932 Adjunctive Generalized Anxiety Disorder	NCT06846320	Temecula
M4OLE	14	NBI-1117568 Schizophrenia Open-Label Extension	NCT07114874	Chino
MOONLIGHT	27	JNJ-89495120 MOONLIGHT-1 MDD	NCT06785012	Temecula
MTORCMDD	26	SPN-821 MDD	NCT07226661	Temecula
NEXTVMAT	13	NBI-1065890 in Tardive Dyskinesia	NCT07365462	Chino
NORLIGHT	02	ACP-211 in MDD (NORLIGHT)	NCT07284667	Chino
RAPBP	18	RAP-219 in Bipolar I	NCT07046494	Chino
RESIDUALTD	15	Valbenazine TD4027 Residual TD	NCT07105111	Chino
SAVITRI3	12	NBI-1065845 (Osavampator) Adjunctive MDD	NCT06963021	Chino
YOUTHLIFT	01	Cariprazine (Vraylar) in Pediatric Bipolar I Depression	NCT04777357	Chino

ICD-10 Cross-Reference Index

Find a study by diagnosis code

ICD-10	DIAGNOSIS	STUDIES
F32.x / F33.x	**Major Depressive Disorder**	02 (NORLIGHT), 04 (CLARITY), 05 (BHVLTS), 06 (BHVMONO), 12 (SAVITRI3), 16 (BUOY1), 17 (BUOYOLE), 24 (KORMDD), 25 (EEG300), 26 (MTORCMDD), 27 (MOONLIGHT), 28 (DREEM3S), 29 (APPROACH), 30 (EMBRACE), 31 (EXTEND)
F31.x	**Bipolar I / II disorder**	01 (YOUTHLIFT pediatric BP-D), 03 (ALTOBIP), 08 (BALSAM adjunctive mania), 18 (RAPBP), 20 (LUMABIP pediatric), 23 (M4BP)
F20.x	**Schizophrenia**	07 (COBENTEEN adolescent), 14 (M4OLE), 19 (LUMAPED pediatric)
G30.x	**Alzheimer's disease (+ F06.0 / F06.2 psychotic)**	09 (ADEPT4), 10 (ADEPT5), 11 (CALM45 agitation)
F43.10	**Post-Traumatic Stress Disorder**	33 (EMPOWER1)
F41.1	**Generalized Anxiety Disorder**	22 (M4GAD)
F50.81	**Binge Eating Disorder**	32 (ENGAGE)
G24.01	**Tardive Dyskinesia**	13 (NEXTVMAT), 15 (RESIDUALTD)
F84.0	**Autism Spectrum Disorder (irritability)**	19 (LUMAPED), 21 (LUMAAUT)
G47.00 / G47.10	**Insomnia / Hypersomnia with MDD**	04 (CLARITY), 28 (DREEM3S)

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